

ENGLISH FOR MEDICAL LABORATORY AND SPECIMEN-SUPPORT TEAMS

Medical Laboratory Technicians English

Clear records. Confirmed messages.



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Eight lessons. Eleven extended conversations.

INSIDE THE BOOK

192 vocabulary entries / 128 reusable phrases
220 extended-dialogue turns / 11 transfer exchanges
Word banks and explained answer keys

Upper-intermediate to advanced (B2-C1). For medical laboratory technicians and support staff communicating about identifiers, specimens, quality status, equipment, reports, and handovers.

2026 edition

Eight scenarios. Eight skills.

Eight laboratory conversations practice exact identifiers, recollection requests, release status, service coordination, realistic timing, report versions, critical notifications, and referral follow-up.

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Preserve the distinction that matters.

Laboratory language links records, people, and decisions. Similar accession numbers are not a verified match. An instrument finishing is not permission to release results. A delivered parcel is not proof that a referral laboratory accepted the individual specimen.

Verify before linking records

Read the reference precisely and follow the laboratory verification process before connecting a caller to case information. Do not correct an identifier by guessing which similar number was intended. Keep an unresolved mismatch explicit.

"I need to verify the requested case before discussing its status."

State operational status without diagnosis

Insufficient quantity concerns the specimen available for testing, not a low patient result. Quality review pending describes release status, not a clinical conclusion. Use the documented message and refer missing technical or clinical questions to the authorized role.

"The lead documented insufficient quantity for Q6 and requested recollection through the clinic."

Correct and confirm the whole message

When a spoken number is repeated incorrectly, interrupt the error clearly, restate the correct value with its unit, request the complete read-back, and confirm accuracy. Receipt and understanding must not be inferred from silence or a vague acknowledgment.

"Correction: six point four millimoles per liter, not six point one. Please read back the complete result."

Hand over ownership, not just a note

A pending referral enquiry needs a named recipient, the exact unresolved question, and the agreed update time. Distinguish parcel delivery from accession acceptance and an update commitment from a result promise.

"Eli has accepted the X8 enquiry and the fifteen-hundred clinic update; no result date is confirmed."

PROFESSIONAL BOUNDARIES

All cases and local procedures are fictional. This is English practice, not medical advice or laboratory operating instruction. Follow applicable privacy rules, validated procedures, professional scope, and authorized escalation. No exercise authorizes specimen collection, testing, instrument operation, result release, diagnosis, or treatment. The critical designation in Unit 7 belongs only to fictional Lumen; it is not a universal threshold. Patient identity and recipient authority must be verified through actual local arrangements.

A314 in the log, A341 in the request

A Read the situation

Resolve an accession-reference mismatch through the approved verification route without guessing identity or disclosing an unverified case.

The receiving log lists accession A314. Clinic caller Mira asks technician Noor about A341. No identity match between the request and the logged case has been established. Noor can check the accession log, but Mira must identify the requested case through the laboratory's privacy and verification arrangements. Similar digits do not establish a transcription error or authorize disclosure. The discussion ends with verification still required, not with a claimed case match.

PEOPLE / Noor: Laboratory technician | Mira: Clinic caller

SLOW DOWN THE IDENTIFIER, NOT THE STANDARD

A caller may expect a quick answer because a reference looks nearly right. Be helpful by naming the verification step and repeating the supplied reference precisely. Do not offer another case as a likely match or let conversational confidence replace privacy checks.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which accession appears in the receiving log?

A X8

B A314

C A341

D Q6

2. Which reference does the caller request?

A H2

B T5

C A341

D A314

3. What has been established about identity?

A No match has been established

B Both references definitely identify one patient

C The clinic made a confirmed typo

D Disclosure is automatically authorized

Answers and explanations: [page 85](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

accession

Laboratory registration of a specimen or request under its identifier.

IN USE check the accession

accession number

Reference assigned to a registered laboratory case or specimen.

IN USE read the accession number

accession log

Record of registered laboratory items.

IN USE consult the accession log

receiving log

Record of items received by the laboratory.

IN USE check the receiving log

requisition

Request document specifying laboratory testing information.

IN USE review the requisition

patient identifier

Detail used under the approved process to identify a patient.

IN USE verify patient identifiers

specimen identifier

Reference linking a specimen to its record.

IN USE confirm the specimen identifier

case match

Established link between a request and the correct case.

IN USE establish the case match

identifier mismatch

Difference between references that should be reconciled.

IN USE report an identifier mismatch

transposed digits

Digits whose positions have been exchanged.

IN USE avoid assuming transposed digits

transcription error

Mistake made while copying information.

IN USE investigate a transcription error

verification

Check establishing required information under the applicable process.

IN USE complete verification

Precision check

A314 and A341 contain the same digits in a different order, but that resemblance does not prove they represent the same case. The log entry and caller request must remain distinct until the required verification establishes a match.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

caller identity

Identity of the person making an enquiry.

IN USE verify caller identity

receiving authority

Permission for a person to receive particular information.

IN USE verify receiving authority

confidentiality

Protection of information from unauthorized disclosure.

IN USE maintain confidentiality

disclosure

Sharing information with a recipient.

IN USE authorize disclosure

minimum necessary information

Information limited to what the applicable purpose and rules require.

IN USE limit information sharing

secure channel

Approved communication route with relevant protection.

IN USE use a secure channel

laboratory information system

System managing laboratory records; abbreviated LIS.

IN USE consult the LIS

audit trail

Record showing actions and changes over time.

IN USE preserve the audit trail

source record

Original or authoritative record for the fact being checked.

IN USE consult the source record

unverified reference

Identifier not yet matched through the required checks.

IN USE retain the unverified reference

clarification request

Question seeking missing or inconsistent information.

IN USE raise a clarification request

identity resolution

Process of reconciling identifiers to the correct case.

IN USE complete identity resolution

Precision check

Do not disclose another accession as a suggested answer to an unverified caller. Repeat the reference the caller supplied, explain that verification is needed, and use the approved route for the necessary identifiers and authority checks.

Phrases you can actually use

C Notice the language / 1 of 2

REPEAT THE REQUEST

"You are asking about A341."

STATE THE LIMIT

"I have not established a verified case match."

NAME THE NEXT STEP

"We need to follow the laboratory's verification process."

REQUEST THE APPROVED ROUTE

"Please use the required verification channel."

READ IT CLEARLY

"That is A, three, four, one."

AVOID GUESSING

"I cannot assume a digit transposition."

PROTECT OTHER RECORDS

"I cannot discuss another case as a possible match."

SEPARATE CHECKS

"Caller identity and receiving authority both need the applicable checks."

As supplied

Preserves the caller's actual reference rather than a guessed correction.

Have not established

States the missing verification without disclosing another case.

Cannot assume

Rejects a shortcut while leaving investigation possible.

Phrases you can actually use

C Notice the language / 2 of 2

CONSULT THE RECORD

"I can check the accession log."

KEEP THE ENQUIRY TRACEABLE

"I will retain the reference exactly as supplied."

AVOID A RESULT CLAIM

"No case-specific status has been confirmed in this exchange."

PRESERVE UNCERTAINTY

"The reference remains unverified."

AVOID AUTOMATIC CORRECTION

"I will not change the reference to make it fit."

EXPLAIN THE PURPOSE

"This helps ensure we discuss the correct case."

REQUEST CLARIFICATION

"Please confirm the requested case through the approved arrangements."

CLOSE PRECISELY

"Verification is still required before case details can be discussed."

Both

Separates identity from authority to receive information.

Before

Makes disclosure dependent on the required verification.

Still required

Prevents the conversation from being mistaken for a completed match.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which response is appropriate before verification?

- A** I will change the log to A341
- B** You asked about A341; please complete the required case verification
- C** I found A314, so I will give you its details
- D** The digits look similar, so this is the same patient

2. What does the digit resemblance establish?

- A** A final result for A341
- B** A possible mismatch requiring checking, not identity
- C** A confirmed typo by the clinic
- D** Automatic disclosure permission

3. Which action preserves traceability?

- A** Keep the supplied reference and document the unresolved enquiry
- B** Replace the caller's number silently
- C** Delete the enquiry because it is inconvenient
- D** Combine both cases without review

4. Which final status is accurate?

- A** A341 is confirmed absent from every system
- B** Identity verification remains required
- C** The case match is complete
- D** The other accession has been disclosed appropriately

[Answers and explanations: page 85.](#)

A314 in the log, A341 in the request

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

accession log / Receiving authority / unverified reference / Confidentiality / case match / secure channel / verification / source record / audit trail / transcription error

- 01 **Mira:** I have A341 on our request. Could you check the case status? I'm calling from the clinic.
- 02 **Noor:** You are asking about A, three, four, one. I need to complete the required ¹ _____ before discussing case details, and I have not established a match yet.
- 03 **Mira:** I may have transposed two digits. Could you tell me which other reference looks closest?
- 04 **Noor:** That is helpful. A possible ² _____ should be checked against the appropriate source rather than assumed. I will retain the number you actually supplied.
- 05 **Mira:** If you can search the log, why can't we just use the closest number and confirm it afterward?
- 06 **Noor:** I can consult the ³ _____. That does not replace the case and recipient checks required by our process, and I cannot offer another case as a likely answer.
- 07 **Mira:** Understood. I'll keep our request separate until the required checks link it to the right case.
- 08 **Noor:** Exactly. The ⁴ _____ remains unresolved. Similar digits are not enough to link records or authorize a case-specific status update.
- 09 **Mira:** I can use the usual protected verification route. Should I send the details through that process rather than this enquiry?
- 10 **Noor:** Yes, use the approved ⁵ _____ and provide the information that process requires. We should not improvise a different identity check during this call.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 10.

- 11 **Mira:** I'll check the original requisition. A number I remember from another call may not be reliable.
- 12 **Noor:** The ⁶ _____ is the right place to confirm what was requested. Please keep any correction traceable through the established arrangements.
- 13 **Mira:** Our clinic name is on the request. Does that also establish my authority to receive this particular information?
- 14 **Noor:** Correct. ⁷ _____ needs the applicable check as well. We should keep the correct case and the permitted recipient clear before sharing its status.
- 15 **Mira:** I'll note that I asked about A341 and that verification is still required. I won't write case confirmed.
- 16 **Noor:** That preserves the ⁸ _____. It also avoids turning a search or clarification conversation into evidence of a completed identity resolution.
- 17 **Mira:** If our source shows a correction, I'll send it through the established route. I won't silently replace the number in the enquiry.
- 18 **Noor:** Thank you. That supports the ⁹ _____ without changing records simply to make two numbers agree.
- 19 **Mira:** I'll complete those checks next. For now, I have no case-specific result or status from you.
- 20 **Noor:** Agreed. ¹⁰ _____ remains important throughout that process. I can assist with the enquiry once the required checks establish the correct case and authorized recipient.

ANSWERS / EXPLANATIONS: PAGE 86

1. verification | 2. transcription error | 3. accession log | 4. case match | 5. secure channel | 6. source record | 7. Receiving authority | 8. unverified reference | 9. audit trail | 10. Confidentiality

Read the reference without guessing

F Say it

Complete the internal training summary. These record references are not authorization to disclose a real case.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

A314 / process / A341 / unverified

Technician: "The receiving log lists ¹ _____."

Caller: "The requested reference is ² _____."

Technician: "The case match remains ³ _____."

Caller: "I will follow the approved verification ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 86

1. A314 | 2. A341 | 3. unverified | 4. process

Rehearse with a fixed script

1. Check the completed dialogue, then read A341 as A, three, four, one. Do not offer the other logged accession as a possible patient match.
2. Switch roles. Repeat the lines that require case verification and receiving authority before disclosure; retain the supplied request unchanged.
3. Complete the four-line exchange. Confirm that A314 belongs to the receiving log and A341 to the caller's request, with the match still unverified.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Insufficient quantity is not a low patient result

A Read the situation

Explain a documented specimen-quantity problem and relay a recollection request without inventing a collection method or clinical interpretation.

The laboratory lead has documented insufficient quantity for test request Q6 and requested recollection through the clinic. No cause, collection method, or urgency is supplied. Clinic coordinator Alex thinks insufficient quantity means the patient has a low test result. Technician Noor corrects that misunderstanding and relays only the documented operational message. Further collection or urgency questions need the appropriate authorized guidance, not a guessed instruction.

PEOPLE / Noor: Laboratory technician | Alex: Clinic coordinator

CORRECT THE MEANING BEFORE ADDING DETAIL

A familiar word such as low can move a conversation from specimen status into an unsupported patient conclusion. State what the term refers to, repeat the documented action, and keep missing clinical or collection details separate.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does the lead document for Q6?

- A Insufficient specimen quantity
- C A confirmed diagnosis

- B A low patient test result
- D A documented collection error

2. What action has the lead requested?

- A Release of a numerical result
- C Immediate treatment

- B Recollection through the clinic
- D A specific unsupplied collection method

3. Which detail is not supplied?

- A The quantity issue
- C Collection method

- B The lead's recollection request
- D Test-request reference Q6

Answers and explanations: page 87.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

specimen

Material submitted for laboratory examination.

IN USE identify the specimen

specimen suitability

Whether the submitted material meets the relevant test requirements.

IN USE assess specimen suitability

insufficient quantity

Available amount below what the requested test requires here.

IN USE document insufficient quantity

quantity not sufficient

Common laboratory expression for insufficient material; abbreviated QNS.

IN USE clarify a QNS message

recollection

Obtaining another specimen under the applicable instructions.

IN USE request recollection

test request

Order identifying the examination being requested.

IN USE quote the test request

specimen volume

Amount of submitted material.

IN USE distinguish specimen volume

analyte

Substance or property being measured by a test.

IN USE name the analyte

measured concentration

Reported amount of an analyte relative to a unit quantity.

IN USE distinguish measured concentration

patient result

Reported finding from the patient's test.

IN USE avoid inventing a patient result

collection method

Approved way the required specimen is obtained.

IN USE confirm the collection method

collection container

Receptacle specified for a specimen under the relevant procedure.

IN USE verify the collection container

Precision check

Insufficient quantity refers to specimen amount for Q6. It is not a numerical test result and does not mean the patient's measured concentration is low. Do not substitute a clinical conclusion for an operational suitability message.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

specimen requirement

Condition the submitted material must meet for a test.

IN USE consult specimen requirements

rejection reason

Documented reason a specimen cannot be accepted for the requested work.

IN USE relay the rejection reason

preanalytical phase

Work before the analytical measurement, including specimen handling.

IN USE discuss a preanalytical issue

aliquot

Portion separated from a specimen for a defined purpose.

IN USE identify an aliquot

hemolysis

Breakdown of red blood cells that can affect specimen assessment.

IN USE report documented hemolysis

clotted specimen

Specimen containing clotting where relevant to suitability.

IN USE describe a documented clotted specimen

contamination

Unwanted material introduced into a specimen.

IN USE investigate documented contamination

stability

Ability of a specimen or analyte to remain suitable under defined conditions.

IN USE check stability requirements

transport condition

Required condition during specimen movement.

IN USE confirm transport conditions

recollection request

Documented request for another specimen.

IN USE relay the recollection request

clinical interpretation

Explanation of a result's meaning for patient care.

IN USE refer clinical interpretation

authorized guidance

Instructions from the role or procedure permitted to provide them.

IN USE obtain authorized guidance

Precision check

The lead requested recollection through the clinic. That request supplies no method, cause, or urgency. Relay the stated action and seek the appropriate authorized guidance for missing details rather than inventing instructions.

Phrases you can actually use

C Notice the language / 1 of 2

IDENTIFY THE REQUEST

"This message concerns test request Q6."

EXPLAIN THE TERM

"It refers to the amount of specimen available for testing."

AVOID INVENTING A VALUE

"No patient result is supplied in this message."

KEEP CAUSE SEPARATE

"The cause is not documented here."

ATTRIBUTE THE FINDING

"The laboratory lead documented insufficient quantity."

CORRECT THE MISUNDERSTANDING

"It does not mean the patient has a low result."

RELAY THE ACTION

"The lead requested recollection through the clinic."

AVOID BLAME

"I cannot attribute this to a collection error."

Refers to

Defines the subject of the quantity statement.

Does not mean

Corrects the specific clinical misunderstanding.

Documented

Attributes the finding to the supplied record.

Phrases you can actually use

C Notice the language / 2 of 2

NAME MISSING INSTRUCTIONS

"No collection method is included in this message."

KEEP URGENCY SEPARATE

"No urgency classification is supplied here."

PRESERVE THE WORDING

"Please retain insufficient quantity in the record."

LIMIT THE HANDOFF

"I am relaying the documented request, not giving a diagnosis."

SEEK THE RIGHT GUIDANCE

"Please obtain the applicable authorized collection instructions."

AVOID A TIMING PROMISE

"I cannot confirm a new report time from this message."

CHECK UNDERSTANDING

"Are we clear that this is a specimen-amount issue?"

CLOSE ACCURATELY

"Q6 needs recollection through the clinic under the appropriate guidance."

Not included

Marks missing instructions without guessing them.

Through the clinic

Preserves the stated route for the recollection request.

Not giving

Separates operational communication from clinical interpretation.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which explanation is accurate?

- A** The test confirms a diagnosis
- B** There is insufficient specimen quantity for Q6, not a low patient result
- C** The patient has a low concentration
- D** The clinic definitely collected incorrectly

2. Which action is documented?

- A** Emergency treatment
- B** Release of an invented value
- C** Recollection through the clinic
- D** A specific unsupplied collection technique

3. Which statement wrongly adds a cause?

- A** The lead documented insufficient quantity
- B** Collection details need authorized guidance
- C** The collector definitely used the wrong container
- D** No cause is supplied

4. How should urgency be described?

- A** Automatically routine
- B** Automatically critical
- C** Already assessed by the caller
- D** Not supplied in this message; follow the applicable escalation arrangements

Answers and explanations: page 87.

Insufficient quantity is not a low patient result

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

specimen volume / recollection / test request / preanalytical phase / authorized guidance / specimen requirement / recollection request / rejection reason / clinical interpretation / patient result

- 01 **Alex:** I'm updating the clinic's Q6 note. It says insufficient quantity. Should I tell the clinician the patient's result is low?
- 02 **Noor:** No. It refers to ¹ _____, not the patient's concentration. The lead documented too little material for the requested test and asked for recollection through the clinic.
- 03 **Alex:** I'll remove low result. Which reference should stay with the message so the team links it to the right order?
- 04 **Noor:** Keep ² _____ Q6 in the note. The quantity message and the requested follow-up both belong to that request.
- 05 **Alex:** The result field is blank. Should I enter zero until the replacement specimen arrives?
- 06 **Noor:** No. No ³ _____ is supplied. A blank result field does not mean the measured value is zero.
- 07 **Alex:** Understood. The action is to obtain another specimen. Can you confirm that is actually what the lead requested?
- 08 **Noor:** Yes, the documented request is ⁴ _____ through the clinic. That is the action supplied, not an instruction to use a particular collection method.
- 09 **Alex:** Someone has suggested the collector used the wrong container. Is that documented anywhere in this message?
- 10 **Noor:** No. The stated ⁵ _____ is insufficient quantity. It does not establish a container error or identify who caused the shortage.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 19.

- 11 **Alex:** So I can report a suitability issue without recording collector error. Is this a problem before the analytical measurement rather than a low concentration?
- 12 **Noor:** Yes. This is a ⁶ _____ issue about the submitted material. It is not a patient finding from an analytical result.
- 13 **Alex:** The clinic will ask whether recollection is routine or urgent. Neither label appears in the message I have.
- 14 **Noor:** Obtain the applicable ⁷ _____ through the actual escalation arrangements. Don't assign a clinical urgency from the quantity wording alone.
- 15 **Alex:** I also need the collection instructions. I should not choose a container or amount simply to complete the note.
- 16 **Noor:** Correct. Check the relevant ⁸ _____ through the authorized procedure. This message does not provide those instructions or a new report time.
- 17 **Alex:** I'll relay Q6, insufficient specimen quantity, and the lead's request to recollect through the clinic. Missing instructions go to the appropriate contact.
- 18 **Noor:** That preserves the ⁹ _____. It also removes the unsupported low-result statement without replacing it with a guessed cause.
- 19 **Alex:** The clinician may still ask what this means for the patient. I'll distinguish that question from the specimen-status message.
- 20 **Noor:** Yes. Refer ¹⁰ _____ to the appropriately qualified professional with the actual clinical context. Our note should preserve the documented limitation and requested follow-up.

ANSWERS / EXPLANATIONS: PAGE 88

1. specimen volume | 2. test request | 3. patient result | 4. recollection | 5. rejection reason | 6. preanalytical phase | 7. authorized guidance | 8. specimen requirement | 9. recollection request | 10. clinical interpretation

Correct the meaning and retain the request

F Say it

Complete the clinic exchange without adding a collection method or patient finding.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

recollection / result / Q6 / quantity

Technician: "The request reference is ¹ _____."

Clinic: "The issue is insufficient specimen ² _____."

Technician: "The lead requested ³ _____ through the clinic."

Clinic: "This is not a low patient ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 88

1. Q6 | 2. quantity | 3. recollection | 4. result

Rehearse with a fixed script

1. Check the answers, then contrast specimen volume with patient result in turns 1-6. Do not read a blank result field as zero.
2. Switch roles and read the recollection message for Q6. Keep collection method, cause and urgency unspecified, as supplied.
3. Complete the four-line exchange. Say insufficient specimen quantity and recollection through the clinic without adding a diagnosis.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

The run is complete; release is not authorized

A Read the situation

Explain the difference between completed analysis, unresolved quality review, and authorized reporting without supplying unsanctioned results.

An analytical run has finished, but its quality-control review remains unresolved. The laboratory lead has not authorized result release. Clinic contact Dana asks technician Noor to describe the results as final because the instrument has completed its work. No patient values, quality findings, or troubleshooting instructions are supplied. Noor must report the actual status and refer the unresolved review to the laboratory lead without relabeling or releasing results.

PEOPLE / Noor: Laboratory technician | Dana: Clinic contact

A COMPLETED STAGE IS NOT A COMPLETED PROCESS

Under time pressure, a caller may use finished to mean ready to report. Repeat the completed stage, then name the outstanding review and authority. This acknowledges progress without allowing the caller to infer a final report that does not exist.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which stage is complete?

- A Authorized result release
- C The analytical run

- B A final clinic report
- D The quality-control review

2. What is the quality-control status?

- A Review unresolved
- C Confirmed failed under supplied criteria

- B Passed and approved
- D Irrelevant because the instrument stopped

3. Which release claim is supported?

- A Review pending means approval with a warning
- C Instrument completion itself authorizes release

- B The lead has not authorized release
- D A clinic request replaces lead authorization

Answers and explanations: [page 89](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

analytical run

Defined set of measurements performed in one testing sequence.

IN USE complete the analytical run

quality control

Checks used to monitor analytical performance; abbreviated QC.

IN USE review quality control

control material

Material with defined characteristics used to assess analytical performance.

IN USE identify control material

control result

Measurement obtained from a quality-control material.

IN USE review a control result

acceptance limit

Defined boundary used to assess an applicable control condition.

IN USE apply the approved acceptance limit

quality review

Authorized assessment of relevant quality evidence.

IN USE await quality review

release authorization

Permission to issue results under the applicable process.

IN USE obtain release authorization

reportable result

Result meeting the applicable requirements for reporting.

IN USE distinguish a reportable result

standard deviation

Measure of spread around the mean; abbreviated SD.

IN USE express a difference in SD units

pending review

Status indicating an assessment is not yet resolved.

IN USE retain pending review

z-score

Difference from a specified mean divided by the specified SD.

IN USE calculate the control z-score

report status

Current stage of a report in its lifecycle.

IN USE state report status

Precision check

Run complete is an instrument-workflow status. It does not establish resolved quality control or release authorization. Because review is unresolved, do not describe the quality outcome as passed, failed, or irrelevant.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

precision

Closeness among repeated measurements under specified conditions.

IN USE discuss analytical precision

accuracy

Closeness of a measurement to an accepted reference value.

IN USE distinguish accuracy from precision

bias

Systematic difference from an appropriate reference.

IN USE investigate documented bias

random variation

Unpredictable spread in measurements under relevant conditions.

IN USE assess random variation

trend

Directional pattern across observations over time.

IN USE identify a control trend

shift

Sustained change in the level of observed measurements.

IN USE investigate a control shift

out-of-control condition

Quality state outside the applicable accepted criteria.

IN USE report a documented out-of-control condition

coefficient of variation

SD divided by the mean, usually expressed as a percentage; CV.

IN USE report the CV with its basis

withheld result

Result not released pending the applicable process.

IN USE preserve withheld-result status

review outcome

Documented conclusion of an assessment.

IN USE request the review outcome

authorized reviewer

Person permitted to assess and approve the relevant evidence.

IN USE consult the authorized reviewer

status update

Message reporting the current process stage.

IN USE give a status update

Precision check

The clinic may need an update, but its request does not supply the missing authorization. Give the current stage and route the review question to the laboratory lead. No values or technical corrective actions are part of this exchange.

Phrases you can actually use

C Notice the language / 1 of 2

03

ACKNOWLEDGE COMPLETION

"The analytical run has finished."

STATE THE AUTHORITY LIMIT

"The lead has not authorized result release."

AVOID AN INVENTED QC OUTCOME

"I cannot describe the review as passed or failed."

KEEP VALUES PROTECTED

"I am not releasing patient values in this exchange."

NAME THE UNRESOLVED STAGE

"Quality-control review remains unresolved."

CORRECT THE INFERENCE

"Instrument completion does not make the report final."

OFFER THE ACTUAL STATUS

"I can provide a process-status update."

REFER THE REVIEW

"The laboratory lead needs to resolve the review."

Remains

Shows that completion of one stage did not resolve another.

Does not make

Blocks the shortcut from instrument status to final report.

Passed or failed

Names two unsupported conclusions, not two available choices.

Phrases you can actually use

C Notice the language / 2 of 2

03

AVOID A DEADLINE PROMISE

"No release time is confirmed."

PRESERVE THE WORDING

"Please retain pending review in the update."

ATTRIBUTE THE NEXT DECISION

"Release depends on the authorized process."

CHECK THE HANDOFF

"Please confirm that final reporting has not been authorized."

SEPARATE STAGES

"Analysis, review, and release are different steps."

RESIST RELABELING

"I cannot relabel unreleased results as final."

AVOID TECHNICAL IMPROVISATION

"I am not supplying troubleshooting instructions."

CLOSE ACCURATELY

"Run complete, review unresolved, release not authorized."

Depends on

Preserves the required authorization relationship.

Not confirmed

Avoids turning a requested deadline into a promise.

Relabel

Identifies a wording change that would misrepresent actual status.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which update is accurate?

- A** The run is complete, QC review is unresolved, and release is not authorized
- B** The report is final because the instrument finished
- C** QC is confirmed failed
- D** The clinic's request authorizes release

2. What does unresolved QC review establish?

- A** No completed review outcome is supplied
- B** Every result is wrong
- C** Every result is final
- D** The review can be ignored

3. Who should address the review question?

- A** The learner by inventing a control limit
- B** The laboratory lead under the authorized process
- C** The caller by renaming the report
- D** An unrelated delivery driver

4. Which action misrepresents status?

- A** Calling the results final without authorization
- B** Reporting that the run finished
- C** Saying review remains unresolved
- D** Declining to invent a release time

Answers and explanations: [page 89](#).

The run is complete; release is not authorized

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

pending review / quality control / reportable result / report status / status update / review outcome / analytical run / withheld result / release authorization / authorized reviewer

- 01 **Dana:** The instrument has finished. Can I tell the clinic these are final results? They're waiting for an answer.
- 02 **Noor:** The ¹ _____ is complete, but quality review remains unresolved. The laboratory lead has not authorized release.
- 03 **Dana:** I'll remove final from the update. What wording should I use for the stage that is still outstanding?
- 04 **Noor:** Use ² _____. That describes the unresolved assessment without suggesting the controls passed or failed.
- 05 **Dana:** I was about to call it a QC failure because nothing is being released. Is that too strong?
- 06 **Noor:** Yes. No ³ _____ is supplied. An unresolved assessment is not the same as a documented failure.
- 07 **Dana:** The clinic still needs a message. Can we give them the process status without including patient values?
- 08 **Noor:** Yes. A ⁴ _____ can say analysis completed, review unresolved, release not authorized. It must not imply a final report exists.
- 09 **Dana:** Would the clinic's request to treat it as final count as permission? They say they accept that review is pending.
- 10 **Noor:** No. ⁵ _____ must come through the laboratory's applicable process. The request does not replace the lead's decision.

The conversation, continued

E Conversations / turns 11-20

03

Continue with the word bank on page 28.

- 11 **Dana:** Then the next question is who can resolve the review, rather than whether I can change the label.
- 12 **Noor:** Correct. The ⁶ _____ needs to assess the relevant evidence. I can pass the enquiry to the laboratory lead without inventing a technical conclusion.
- 13 **Dana:** If I leave final off but say available, could that still suggest the results are ready to use?
- 14 **Noor:** It could. Keep the ⁷ _____ explicit: not released, not final. A softer label should not hide the outstanding requirement.
- 15 **Dana:** Understood. The completed measurement stage alone does not establish that the result meets the reporting requirements.
- 16 **Noor:** Exactly. A ⁸ _____ requires those requirements to be met; instrument completion alone is not enough.
- 17 **Dana:** Is there any supplied indication that the review can be skipped because the clinic is under time pressure?
- 18 **Noor:** No. The unresolved ⁹ _____ remains part of the process. We have no authorization to bypass it or provide troubleshooting instructions.
- 19 **Dana:** I'll send the process update, remove the final claim, and direct the release-status enquiry to the lead.
- 20 **Noor:** Good. Keep any ¹⁰ _____ unreleased until the applicable review and authorization are resolved. A clear update can acknowledge the delay without supplying unsanctioned findings.

ANSWERS / EXPLANATIONS: PAGE 90

1. analytical run | 2. pending review | 3. review outcome | 4. status update | 5. release authorization | 6. authorized reviewer | 7. report status | 8. reportable result | 9. quality control | 10. withheld result

Three stages, three separate facts

F Say it

03

Complete the clinic update using the actual run, review, and release status.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

final / unresolved / authorized / complete

Technician: "The analytical run is ¹ _____."

Clinic: "Quality-control review remains ² _____."

Technician: "Result release is not ³ _____."

Clinic: "We must not call the report ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 90

1. complete | 2. unresolved | 3. authorized | 4. final

Rehearse with a fixed script

1. After checking the dialogue, read the three statuses aloud: analysis complete, quality review unresolved, release not authorized.
2. Switch roles. Correct final to not released in the printed exchange; keep unresolved distinct from both QC passed and QC failed.
3. Complete the four-line exchange. Retain the lead's authorization requirement and do not substitute the clinic's request for permission.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

The service visit ended; H2 is still unavailable

04

A Read the situation

Request a documented instrument-status decision and distinguish service attendance from authorized return to use.

Analyzer H2 is marked unavailable. A service visit ended at 11:00, but the laboratory lead has not confirmed return to service. Technician Noor asks lead Elena for the status needed before updating the clinic. No repair steps, instrument settings, reagent findings, patient results, or confirmed restart time are supplied. The end of the service visit is a known event, not evidence that the analyzer is available.

PEOPLE / Noor: Laboratory technician | Elena: Laboratory lead

NAME THE STATUS, NOT THE HOPEFUL IMPLICATION

A completed visit can sound like a completed repair and approved restart. Keep those events separate. Ask the lead for the documented operating status rather than interpreting a service departure time as permission to use equipment.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is H2's current recorded status?

A Unavailable

B Returned to service

C Approved for all testing

D Confirmed beyond repair

2. What happened at 11:00?

A The service visit ended

B H2 was authorized for use

C All patient results were released

D A restart was guaranteed

3. What does Noor need before the clinic update?

A The lead's confirmed status

B A guessed restart time

C Unprovided repair instructions

D An invented reagent diagnosis

Answers and explanations: page 91.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

analyzer

Instrument used to perform laboratory measurements.

IN USE identify the analyzer

instrument status

Recorded condition or availability of equipment.

IN USE confirm instrument status

unavailable

Not available for the stated operational use.

IN USE retain unavailable status

service visit

Attendance by personnel providing equipment support.

IN USE record the service visit

return to service

Authorized restoration of equipment to operational use.

IN USE confirm return to service

service report

Record describing work or findings from a service event.

IN USE review the service report

maintenance log

Record of equipment upkeep and related actions.

IN USE consult the maintenance log

equipment identifier

Reference distinguishing one instrument from another.

IN USE quote the equipment identifier

downtime

Period when equipment is not available for its intended use.

IN USE report downtime

restart authorization

Permission to resume operation under the relevant process.

IN USE obtain restart authorization

reagent

Substance used in a laboratory reaction or analytical process.

IN USE identify the reagent

reagent lot

Identified production batch of a reagent.

IN USE record the reagent lot

Precision check

The service visit ended at 11:00, but H2 remains marked unavailable. A departure time is not return-to-service authorization, proof of a completed repair, or a guaranteed restart time.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

lot number

Reference identifying a particular production batch.

IN USE verify the lot number

expiry date

Date limiting use under applicable product instructions.

IN USE check the expiry date

inventory status

Recorded availability of supplies.

IN USE check inventory status

calibration

Establishing the relation between instrument response and reference values.

IN USE document calibration

verification check

Check confirming a specified requirement is met.

IN USE record a verification check

preventive maintenance

Planned upkeep intended to reduce equipment problems.

IN USE schedule preventive maintenance

corrective maintenance

Work addressing an identified equipment problem.

IN USE document corrective maintenance

service provider

Organization or person supplying equipment support.

IN USE contact the service provider

operational readiness

State meeting the requirements for intended operation.

IN USE confirm operational readiness

status confirmation

Explicit verification of the current equipment condition.

IN USE request status confirmation

outstanding review

Assessment not yet completed or resolved.

IN USE identify outstanding review

dilution factor

Ratio of final diluted volume to the original sample volume.

IN USE verify whether the factor was applied

Precision check

Request the lead's confirmed status and report only the authorized message. Do not add repair steps, settings, calibration results, reagent causes, or patient findings that are not supplied in the record.

Phrases you can actually use

C Notice the language / 1 of 2

IDENTIFY THE INSTRUMENT

"I am checking the status of analyzer H2."

NAME THE KNOWN EVENT

"The service visit ended at eleven."

ASK FOR AUTHORITY

"Has the lead confirmed operational availability?"

AVOID A RESTART PROMISE

"I have no confirmed restart time."

STATE THE CURRENT RECORD

"H2 is marked unavailable."

SEPARATE THE IMPLICATION

"That does not confirm return to service."

REQUEST THE RECORD

"What status is documented for the clinic update?"

KEEP REPAIR FINDINGS SEPARATE

"The visit end does not tell me what repair was completed."

Marked

Attributes status to the record rather than a personal impression.

Ended at

Gives an event time without implying operational approval.

Does not confirm

Separates evidence of attendance from evidence of readiness.

Phrases you can actually use

C Notice the language / 2 of 2

AVOID INVENTING A REAGENT ISSUE

"No reagent finding is supplied here."

PRESERVE THE EQUIPMENT REFERENCE

"Please keep H2 in the status message."

LIMIT THE CLINIC MESSAGE

"I will report the current documented availability."

REQUEST EXPLICIT NOTIFICATION

"Please notify me when the authorized status changes."

AVOID TECHNICAL IMPROVISATION

"I am not changing settings based on this conversation."

NAME THE PENDING STEP

"Return-to-service confirmation is still outstanding."

SEPARATE SERVICE AND TESTING

"A service visit is not permission to resume analysis."

CLOSE PRECISELY

"Visit ended at eleven; H2 remains unavailable pending confirmation."

Has ... confirmed

Requests a completed authorized decision.

No ... supplied

Keeps technical findings outside the stated evidence.

When ... changes

Makes a later update conditional on an actual status change.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which clinic update is supported?

- A** H2 remains unavailable; the service visit ended at 11:00 but return is unconfirmed
- B** H2 is ready because the engineer left
- C** All results will be final at 11:00
- D** The reagent lot caused the outage

2. What would establish a status change here?

- A** The laboratory lead's confirmed return-to-service status
- B** The visit end alone
- C** A caller's assumption
- D** The learner choosing a setting

3. Which detail must not be invented?

- A** The visit's 11:00 end
- B** The unavailable label
- C** A restart time
- D** The identifier H2

4. What does the service visit prove?

- A** Patient results are authorized
- B** Support personnel attended and the visit ended
- C** Every repair was successful
- D** Calibration passed

Answers and explanations: [page 91](#).

The service visit ended; H2 is still unavailable

04

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

Return to service / downtime / equipment identifier / Calibration / instrument status / status confirmation / Operational readiness / service report / service visit / reagent lot

- 01 **Noor:** The clinic saw that service on H2 ended at eleven. They're asking if we can use the analyzer again. Its status still says unavailable.
- 02 **Elena:** Keep the ¹ _____ separate from the visit time. I have not confirmed return to service, so unavailable remains the status you can report.
- 03 **Noor:** Does the completed visit establish anything about availability, or only that the service attendance has finished?
- 04 **Elena:** Correct. A ² _____ can end without establishing operational readiness. We need the relevant documented outcome and authorized decision, not an inference from departure.
- 05 **Noor:** I'll remove eleven as the restart time. Can you confirm that return to use is still outstanding?
- 06 **Elena:** Exactly. ³ _____ has not been confirmed. Please do not turn a known event time into a commitment we have not made.
- 07 **Noor:** Which document should the authorized reviewer check for the work completed? I don't have the service findings.
- 08 **Elena:** The ⁴ _____ is relevant to the authorized review, but do not invent its contents. No repair steps or completed verification findings are supplied in this exchange.
- 09 **Noor:** The clinic is asking if this was a reagent problem. I don't see a reagent batch or fault finding in the supplied information.
- 10 **Elena:** Good. A ⁵ _____ reference would be a specific technical claim. We have no such finding here, and it should not be added to make the explanation sound complete.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 37.

- 11 **Noor:** Someone has also described calibration as passed. We don't have that result from the completed visit, do we?
- 12 **Elena:** Correct. ⁶ _____ status and settings belong to the applicable technical process. This conversation is about accurate coordination, not an instruction to perform or bypass that work.
- 13 **Noor:** Then my update is H2 unavailable, service visit ended at eleven, and no confirmed restart time. I won't give a duration for the remaining outage.
- 14 **Elena:** That describes the ⁷ _____ without inventing its cause or duration. We can acknowledge the completed visit while preserving the unresolved operating status.
- 15 **Noor:** If the next message just says service done, I'll still need to ask whether the operational status changed.
- 16 **Elena:** Yes. Ask for ⁸ _____ tied to H2. The wording should tell you whether availability changed, not merely that a person completed a task.
- 17 **Noor:** I'll retain H2 in the subject and message. An update about another instrument would not resolve this enquiry.
- 18 **Elena:** The ⁹ _____ should remain visible. That is especially important when multiple service enquiries are being handled at once.
- 19 **Noor:** Please send the documented status change through the proper route when available. Until then, I'll keep the clinic's message at unavailable.
- 20 **Elena:** That is accurate. ¹⁰ _____ remains unconfirmed. I will communicate a documented status change through the proper route when one is available.

ANSWERS / EXPLANATIONS: PAGE 92

1. instrument status | 2. service visit | 3. Return to service | 4. service report | 5. reagent lot | 6. Calibration | 7. downtime | 8. status confirmation | 9. equipment identifier | 10. Operational readiness

Do not turn a visit time into a restart time

F Say it

04

Complete the status exchange using the known event and unresolved availability.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

unavailable / H2 / confirmed / 11:00

Technician: "The analyzer identifier is ¹ _____."

Lead: "Its recorded status remains ² _____."

Technician: "The service visit ended at ³ _____."

Lead: "Return to service is not yet ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 92

1. H2 | 2. unavailable | 3. 11:00 | 4. confirmed

Rehearse with a fixed script

1. Check the dialogue and read H2 unavailable beside service visit ended at eleven. Stress which statement describes the instrument.
2. Switch roles. Read the lines rejecting an unsupported restart time, reagent fault and passed calibration; retain the missing findings.
3. Complete the four-line exchange. End with return to service unconfirmed, not operational readiness established.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

The two-hour clock starts at receipt

A Read the situation

Clarify a turnaround-time starting point and distinguish a quoted interval from a confirmed reporting promise during a backlog.

05

The service quotes a two-hour turnaround measured from laboratory receipt to reporting. Specimen T5 was collected at 08:00 and received by the laboratory at 09:00. A backlog affects estimates, and no report time is confirmed. Clinic coordinator Eva assumes the two hours began at collection and expects a report at 10:00. Technician Noor explains the receipt-based interval without replacing the mistaken 10:00 promise with an unsupported 11:00 guarantee.

PEOPLE / Noor: Laboratory technician | Eva: Clinic coordinator

CORRECT THE CLOCK AND THE CERTAINTY LEVEL

A timing misunderstanding may involve both the starting event and the strength of a promise. Explain each separately. A calculation from a quoted interval is not the same as a confirmed delivery time, especially when a stated backlog affects estimates.

Briefing checks

Circle one answer in each item. Use the briefing.

1. When does the quoted turnaround begin?

A Specimen collection

B The clinic's phone call

C The patient's appointment booking

D Laboratory receipt

2. When was T5 received?

A 08:00

B 10:00

C 11:00

D 09:00

3. What report time is confirmed?

A None

B 10:00

C 11:00

D Exactly two hours after this call

Answers and explanations: [page 93](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

turnaround time

Interval between defined starting and ending events; abbreviated TAT.

IN USE clarify turnaround time

collection time

Recorded time the specimen was obtained.

IN USE distinguish collection time

receipt time

Recorded time the laboratory received the specimen.

IN USE confirm receipt time

reporting time

Time a report is issued under the relevant process.

IN USE confirm reporting time

start point

Event from which an interval is measured.

IN USE name the start point

end point

Event at which a measured interval ends.

IN USE define the end point

quoted interval

Stated expected duration under the service description.

IN USE explain the quoted interval

estimate

Approximate expectation subject to relevant uncertainty.

IN USE qualify the estimate

confirmed deadline

Explicitly established time commitment.

IN USE distinguish a confirmed deadline

backlog

Accumulated work awaiting completion.

IN USE explain the backlog

work queue

Ordered or managed set of pending tasks.

IN USE check the work queue

elapsed time

Time that has passed since a specified event.

IN USE calculate elapsed time

Precision check

Collection at 08:00 and receipt at 09:00 are different events. Applying the quoted two-hour interval to receipt gives 11:00 as the arithmetic reference, not 10:00. It still does not confirm a report time.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

transport interval

Time between the relevant dispatch and receipt events.

IN USE distinguish the transport interval

receipt-to-report interval

Duration measured from laboratory receipt to reporting.

IN USE explain the receipt-to-report interval

service standard

Defined service expectation under its stated conditions.

IN USE clarify the service standard

estimated completion

Predicted finish time rather than a confirmed outcome.

IN USE qualify estimated completion

delay notice

Communication explaining a later-than-expected process.

IN USE issue a delay notice

update commitment

Promise to communicate status at a stated time.

IN USE separate an update commitment

result commitment

Promise that a result will be available at a stated time.

IN USE avoid an unsupported result commitment

queue status

Current position or condition of pending work.

IN USE report queue status

time reference

Stated event and clock time used in a message.

IN USE preserve the time reference

expectation reset

Correction of an earlier timing assumption.

IN USE make an expectation reset

escalation route

Authorized path for raising a concern requiring further attention.

IN USE use the escalation route

confirmed information

Details established by the relevant record or authority.

IN USE relay confirmed information

Precision check

The backlog affects estimates, so do not promise 11:00 merely because the starting point is now correct. Keep any status-update commitment separate from a result promise, and use the actual escalation route for clinical urgency concerns.

Phrases you can actually use

C Notice the language / 1 of 2

IDENTIFY THE SPECIMEN

"This timing enquiry concerns T5."

STATE RECEIPT

"Laboratory receipt was recorded at nine."

CORRECT THE STARTING POINT

"Collection does not start this service's quoted clock."

LIMIT THE CALCULATION

"That is not a confirmed reporting promise."

STATE COLLECTION

"Collection was recorded at eight."

DEFINE THE SERVICE INTERVAL

"The quoted two hours run from receipt to reporting."

EXPLAIN THE ARITHMETIC

"Nine plus two hours gives eleven as the reference calculation."

NAME THE DELAY FACTOR

"A backlog is affecting estimates."

From ... to

Defines both ends of the measured interval.

Reference calculation

Separates arithmetic from an actual service commitment.

Affecting estimates

Explains why the quoted duration does not settle the current case.

Phrases you can actually use

C Notice the language / 2 of 2

PRESERVE UNCERTAINTY

"No report time is confirmed."

SEPARATE AN UPDATE

"A status update is not a promise that results will be ready."

KEEP THE CLINICAL BOUNDARY

"Urgency concerns need the applicable clinical escalation route."

CORRECT THE CLINIC MESSAGE

"Please state receipt-based turnaround and the current uncertainty."

AVOID REPLACING ONE PROMISE

"I cannot change an unsupported ten-o'clock promise into an eleven-o'clock guarantee."

ASK FOR THE RIGHT INFORMATION

"We need the current authorized estimate, if available."

AVOID REASSURANCE WITHOUT EVIDENCE

"I cannot promise the delay is clinically unimportant."

CLOSE WITH THE FACTS

"Collected at eight, received at nine, reporting time unconfirmed."

If available

Avoids implying a current estimate already exists.

Not a promise

Limits the meaning of a status update.

Current uncertainty

Keeps the unresolved reporting time explicit after correcting the starting point.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Why is the assumed 10:00 time unsupported?

- A T5 was received at ten
- B The service quotes three hours
- C Collection and receipt are always simultaneous
- D It starts the two-hour interval at collection rather than receipt

2. What does 09:00 plus two hours establish?

- A Proof that no backlog exists
- B An 11:00 reference calculation, not a confirmed report time
- C Guaranteed final results at eleven
- D A new clinical urgency classification

3. Which update is accurate?

- A T5 arrived at nine; backlog affects estimates; no report time is confirmed
- B T5 is guaranteed final at ten
- C T5 is guaranteed final at eleven
- D The delay has no possible clinical importance

4. What is distinct from a result commitment?

- A A guaranteed final report deadline
- B A confirmed reporting time
- C An agreed time to provide a status update
- D An explicit promise results will be ready

Answers and explanations: [page 93](#).

The two-hour clock starts at receipt

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

05

WORD + PHRASE BANK

update commitment / confirmed deadline / receipt time / turnaround time / time reference / expectation reset / quoted interval / escalation route / backlog / reference calculation

- 01 **Eva:** I told the clinic T5 would be reported at ten: collected at eight plus the quoted two hours. Have I used the right starting point?
- 02 **Noor:** The quoted ¹ _____ starts at laboratory receipt, not collection. T5 was received at nine, and a backlog is currently affecting estimates.
- 03 **Eva:** Then the nine-o'clock laboratory entry matters for that interval. I'll keep the eight-o'clock collection event in the record as a separate fact.
- 04 **Noor:** Correct. The ² _____ is the relevant starting point here. It is important to keep both events in the record rather than replacing one with the other.
- 05 **Eva:** Nine plus two gives eleven. Is eleven an actual reporting commitment, or only what the quoted interval would calculate?
- 06 **Noor:** No. That is a ³ _____, not a confirmed reporting promise. No report time is confirmed, and the backlog means the estimate cannot be treated as guaranteed.
- 07 **Eva:** I need to withdraw the guarantee as well as correct the arithmetic. Simply replacing ten with eleven would keep the certainty error.
- 08 **Noor:** Exactly. The ⁴ _____ explains how timing is described, but it does not by itself establish when this particular report will be issued under the current conditions.
- 09 **Eva:** I'll tell the clinic I used collection instead of receipt and gave a promise the available status didn't support.
- 10 **Noor:** That is a clear ⁵ _____. It preserves the known collection and receipt facts while withdrawing the unsupported ten-o'clock commitment.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 46.

- 11 **Eva:** Can I attribute the uncertainty to a confirmed backlog, or would that be another assumption?
- 12 **Noor:** The ⁶ _____ is a stated factor affecting estimates. We should not add an exact queue position, duration, or cause beyond the information actually available.
- 13 **Eva:** If we later agree an update time, I'll say update rather than report ready. No replacement update time has been agreed here.
- 14 **Noor:** Yes. An ⁷ _____ is different from a result commitment. No new update time is supplied in this discussion, so we should not invent one either.
- 15 **Eva:** If the clinic raises clinical urgency, whom should that concern go through? I can't assess it from these timestamps.
- 16 **Noor:** Use the applicable ⁸ _____. We must not dismiss urgency or give clinical reassurance merely because the quoted interval was misunderstood.
- 17 **Eva:** My correction will retain collection at eight, receipt at nine, and the quoted two-hour receipt-to-report interval, with no report time confirmed.
- 18 **Noor:** That keeps the ⁹ _____ explicit. Anyone reading the note can see which event starts the quoted interval and which timing remains unknown.
- 19 **Eva:** I'll withdraw ten o'clock and avoid promising eleven. Any actual reporting commitment needs its own confirmation.
- 20 **Noor:** Agreed. A ¹⁰ _____ is not supplied here. Keep the calculation, estimate, and any later authorized commitment distinct in subsequent messages.

ANSWERS / EXPLANATIONS: PAGE 94

1. turnaround time | 2. receipt time | 3. reference calculation | 4. quoted interval | 5. expectation reset | 6. backlog | 7. update commitment | 8. escalation route | 9. time reference | 10. confirmed deadline

Correct the clock without inventing a promise

F Say it

Complete the T5 timing summary. Clock values identify known events or arithmetic only.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

11:00 / 09:00 / unconfirmed / 08:00

Clinic: "T5 was collected at ¹ _____."

Technician: "Laboratory receipt was at ² _____."

Clinic: "Adding the quoted two hours gives ³ _____ as a reference calculation."

Technician: "The actual report time remains ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 94

1. 08:00 | 2. 09:00 | 3. 11:00 | 4. unconfirmed

Rehearse with a fixed script

1. Check the answers. Read collection at eight and receipt at nine as separate events, then calculate nine plus two as eleven.

2. Switch roles. Read the correction withdrawing the ten-o'clock promise; do not replace it with an eleven-o'clock guarantee.

3. Complete the four-line exchange. Keep a status update separate from a report-ready commitment, and retain the unconfirmed reporting time.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Correcting the ten-o'clock report label

A Read the situation

Withdraw an inaccurate report-status statement and identify the superseding final version without inventing changed findings or an amendment.

The clinic received a preliminary report at 10:00 and a final report at 14:00 for the same request. Technician Noor mistakenly called the earlier version final. Clinic contact Priya asks which report status is correct. The 14:00 final report supersedes the 10:00 preliminary version for this request. No findings, clinical conclusions, or subsequent amended report are supplied. Noor needs to correct the earlier communication clearly and retain a traceable record.

PEOPLE / Noor: Laboratory technician | Priya: Clinic contact

NAME THE CORRECTION DIRECTLY

A vague later update may leave the original mistaken statement in circulation. Identify the earlier label, withdraw it explicitly, and state the correct version and time. Correcting communication is not the same as claiming the laboratory issued an amended clinical report.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What was received at 10:00?

- | | |
|-------------------------------|-------------------------------------|
| A A preliminary report | B The final report |
| C An amended report | D A confirmed diagnosis only |

2. Which version supersedes it?

- | | |
|------------------------------------|---------------------------------------|
| A The 14:00 final report | B An unspecified future report |
| C The incorrect phone label | D Neither version |

3. What has not been supplied?

- | | |
|---|---------------------------------------|
| A The preliminary status at ten | B The final status at fourteen |
| C Findings or a later amended report | D The two issue times |

Answers and explanations: [page 95](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

preliminary report

Report issued before the relevant final reporting stage.

IN USE identify the preliminary report

final report

Report issued with final status under the applicable process.

IN USE confirm the final report

amended report

Formally revised report issued through the applicable correction process.

IN USE distinguish an amended report

supersede

Replace an earlier version as the applicable later record.

IN USE supersede the preliminary version

report version

Identified edition or stage of a report.

IN USE check the report version

issue time

Recorded time a report was issued.

IN USE quote the issue time

status label

Wording identifying a report's current stage.

IN USE correct the status label

communication correction

Explicit repair of an inaccurate spoken or written message.

IN USE make a communication correction

detection limit

Lowest amount reliably detectable under the stated method conditions.

IN USE distinguish detection from quantification

version control

Management of identifiable document revisions and their relationships.

IN USE preserve version control

current report

Report currently applicable under the stated reporting process.

IN USE identify the current report

superseded version

Earlier record replaced by an applicable later version.

IN USE retain the superseded version appropriately

Precision check

The 10:00 report was preliminary; calling it final was a communication error. The 14:00 final report supersedes that version. This sequence does not establish what any findings say or whether their content changed.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

correction trail

Record linking an error to its documented correction.

IN USE preserve the correction trail

recipient acknowledgment

Confirmation that the intended recipient received the message.

IN USE obtain recipient acknowledgment

report discrepancy

Difference in report information or status requiring clarification.

IN USE clarify the report discrepancy

finding

Recorded result or observation in a report.

IN USE avoid inventing a finding

interpretation

Explanation of the meaning of a finding.

IN USE refer interpretation appropriately

addendum

Additional report information issued under the applicable process.

IN USE distinguish an addendum

revision reason

Documented explanation for a report change.

IN USE state the revision reason

reissue

Issuance of a report again under the applicable procedure.

IN USE verify the reissue status

distribution record

Record of where a report or update was sent.

IN USE check the distribution record

document history

Record of earlier versions and changes.

IN USE preserve document history

explicit correction

Statement directly identifying and fixing the prior error.

IN USE provide an explicit correction

quantification limit

Lowest amount quantified with suitable precision and accuracy.

IN USE retain the below-quantification qualifier

Precision check

An amended report is a formally revised report under the relevant process. Correcting Noor's inaccurate label does not prove such a report exists. Preserve the report history and follow the actual record-correction arrangements rather than deleting the earlier version.

Phrases you can actually use

C Notice the language / 1 of 2

OPEN THE CORRECTION

"I need to correct my earlier description."

WITHDRAW THE LABEL

"That label was incorrect."

IDENTIFY THE LATER VERSION

"The fourteen-hundred report is final."

AVOID A CONTENT CLAIM

"I am not saying the findings changed."

NAME THE ERROR

"I called the ten-o'clock report final."

STATE THE CORRECT STATUS

"The ten-o'clock report was preliminary."

EXPLAIN THE RELATIONSHIP

"The final report supersedes the preliminary version for this request."

AVOID INVENTING AN AMENDMENT

"No later amended report is supplied here."

06

I called

Takes responsibility for the inaccurate description.

Was incorrect

Withdraws the earlier label without ambiguity.

Supersedes

Explains the relationship between versions, not the meaning of findings.

Phrases you can actually use

C Notice the language / 2 of 2

SEPARATE PROCESSES

"This is a correction to my communication."

PRESERVE TIME REFERENCES

"Keep ten and fourteen attached to their respective versions."

IDENTIFY THE CURRENT REFERENCE

"Please use the final version's status in this enquiry."

MAKE FOLLOW-UP TRACEABLE

"Link the correction to the earlier message."

REQUEST ACKNOWLEDGMENT

"Please confirm you have the corrected status."

MAINTAIN HISTORY

"Retain the earlier version according to the record policy."

AVOID CLINICAL INTERPRETATION

"No clinical conclusion is part of this correction."

CLOSE PRECISELY

"Ten was preliminary; fourteen is final and supersedes it."

06

Not saying

Prevents a status correction from becoming a content claim.

No later ... supplied

Limits the known report sequence.

Respective

Keeps each time paired with its own version.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which correction is clear?

- A** The patient findings definitely changed
- B** I incorrectly called the 10:00 report final; it was preliminary; the 14:00 report is final
- C** There was some confusion, but all versions are final
- D** Ignore the times

2. What does supersedes mean here?

- A** Both reports become preliminary
- B** Every previous record must be destroyed
- C** A diagnosis has been reversed
- D** The later final version replaces the preliminary version for this request

3. Which claim is unsupported?

- A** Noor's earlier label was wrong
- B** An amended report was issued after 14:00
- C** The 10:00 report was preliminary
- D** The 14:00 report is final

4. What should be preserved?

- A** An invented new issue time
- B** A claim that no error occurred
- C** A traceable correction and report history under policy
- D** Only the incorrect phone message

Answers and explanations: [page 95](#).

Correcting the ten-o'clock report label

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

finding / status label / issue time / current report / communication correction / document history / amended report / receipt / correction trail / explicit correction

06

- 01 **Noor:** Priya, I need to correct my earlier message. I called the ten-o'clock report final. It was preliminary; the fourteen-hundred report is final.
- 02 **Priya:** Thank you for the ¹ _____. I'll correct my note before passing the earlier label on.
- 03 **Noor:** Please keep both issue times. The ten-o'clock document wasn't final at issue, even though a final version now exists.
- 04 **Priya:** I'll change the earlier ² _____ to preliminary and identify fourteen hundred as the final version. I won't relabel the original document itself.
- 05 **Noor:** Right. I'm correcting what I said about its status, not announcing a new laboratory finding.
- 06 **Priya:** So this is a ³ _____. It repairs your message but does not, by itself, tell me that any clinical content changed.
- 07 **Noor:** Exactly. Nor should we call the fourteen-hundred final report amended merely because I used the wrong word earlier.
- 08 **Priya:** An ⁴ _____ would need its own formal status and record. The supplied sequence is preliminary followed by final.
- 09 **Noor:** Can you repeat the two versions back? I want to make sure the earlier mistake is not carried forward.
- 10 **Priya:** Ten hundred: preliminary. Fourteen hundred: final. I'll retain each ⁵ _____ beside the corresponding status.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 55.

- 11 **Noor:** That's correct. We haven't been given the clinical content of either version, so I can't describe a change in the patient's result.
- 12 **Priya:** I won't infer a changed ⁶ _____ from the status change. The supplied evidence concerns the report versions and your earlier wording.
- 13 **Noor:** Please link this correction to the inaccurate message rather than silently replacing it. The next reader should know what was corrected.
- 14 **Priya:** I'll preserve the ⁷ _____ through the applicable record process. That keeps the original error and its correction traceable.
- 15 **Noor:** The earlier report also belongs in the retained version record under policy. Superseded does not mean erase the history.
- 16 **Priya:** Understood. The ⁸ _____ stays intact while the fourteen-hundred final version is identified as current for this request.
- 17 **Noor:** Can you confirm that you've received the corrected version information, rather than only seen that I sent a follow-up?
- 18 **Priya:** I confirm ⁹ _____: ten hundred preliminary, fourteen hundred final, and your earlier final label for ten hundred is withdrawn.
- 19 **Noor:** Thank you. No later amendment is established here. I'll use the same version wording in any onward communication.
- 20 **Priya:** I'll refer to the fourteen-hundred final version as the ¹⁰ _____, without inventing findings or hiding the earlier correction.

06

ANSWERS / EXPLANATIONS: PAGE 96

1. explicit correction | 2. status label | 3. communication correction | 4. amended report | 5. issue time | 6. finding | 7. correction trail | 8. document history | 9. receipt | 10. current report

Correct the version, not the patient finding

F Say it

Complete the report-status correction using only the supplied times and relationship.

Complete the four gaps. Use each bank entry once.

06

WORD + PHRASE BANK

14:00 / preliminary / supersedes / amended

Technician: "The 10:00 report was ¹ _____."

Clinic: "The ² _____ report is final."

Technician: "The final report ³ _____ the earlier version."

Clinic: "No later ⁴ _____ report is supplied."

ANSWERS / EXPLANATIONS: PAGE 96

1. preliminary | 2. 14:00 | 3. supersedes | 4. amended

Rehearse with a fixed script

1. After checking the answers, read the explicit correction: ten hundred preliminary, fourteen hundred final.
2. Switch roles and repeat both versions and times. Say communication correction without converting the sequence into an amended-report claim.
3. Complete the four-line exchange. Keep the earlier mistake traceable and identify the current final version without inventing clinical findings.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Six point four, not six point one

A Read the situation

Deliver and correct a critical-result message with an exact value, unit, complete read-back, and confirmed receipt.

Fictional Lumen laboratory designates C7's potassium result of 6.4 mmol/L as critical under its own criteria. The unit mmol/L means millimoles per liter. Case identity and Dr Shah's receiving authority are already verified. The report was issued at 14:10, and the call occurs at 14:12. Dr Shah is at extension 214 and initially repeats 6.1. Technician Noor must correct the value, obtain a complete read-back, and confirm receipt. No diagnosis, treatment, or universal critical threshold is supplied.

PEOPLE / Noor: Laboratory technician | Dr Shah: Verified authorized recipient

07

INTERRUPT AN INACCURATE REPEAT CLEARLY

Politeness should not obscure a wrong number. Say correction, give the exact value and unit, and ask for the complete message again. This scenario begins after identity and receiving authority have been verified; the abbreviated case label does not replace real patient-identification requirements.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What potassium value must be communicated?

A 14.10 mmol/L

B 214 mmol/L

C 6.4 mmol/L

D 6.1 mmol/L

2. What is already verified?

A A treatment plan

B A universal critical threshold

C A diagnosis

D Case identity and Dr Shah's receiving authority

3. Which time is the call time?

A 14:10

B 02:14

C 06:40

D 14:12

Answers and explanations: [page 97](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

critical result

Result requiring urgent notification under the laboratory's defined criteria.

IN USE communicate a critical result

critical notification

Communication of a locally designated critical result.

IN USE document the critical notification

potassium

Chemical element measured as an analyte in this fictional report.

IN USE state the potassium result

millimoles per liter

Concentration unit written mmol/L.

IN USE say millimoles per liter

numerical value

Number reported with the relevant analyte and unit.

IN USE correct the numerical value

unit of measurement

Scale or quantity expression accompanying a value.

IN USE retain the unit of measurement

decimal point

Mark separating the whole and fractional parts of a number.

IN USE pronounce the decimal point

read-back

Recipient's spoken repetition of the received message.

IN USE obtain a complete read-back

closed-loop communication

Exchange in which the recipient repeats and sender confirms the message.

IN USE complete closed-loop communication

acknowledgment

Explicit confirmation that a message was received.

IN USE obtain acknowledgment

authorized recipient

Person verified as permitted to receive the information.

IN USE identify the authorized recipient

verified identity

Identity established through the required checks.

IN USE preserve verified identity

Precision check

The correct value is 6.4 mmol/L, spoken six point four millimoles per liter. The incorrect 6.1 read-back must be corrected immediately and followed by a complete accurate repetition, not a vague yes or okay.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

report issue time

Time at which the report was issued.

IN USE state the report issue time

notification time

Time at which the communication occurs.

IN USE record the notification time

contact extension

Internal telephone number for the recipient.

IN USE confirm the contact extension

discrepant repeat

Read-back differing from the intended message.

IN USE correct a discrepant repeat

verbal correction

Spoken statement replacing inaccurate information.

IN USE make a verbal correction

complete message

Required case reference, analyte, value, unit, and relevant status details.

IN USE repeat the complete message

local critical criteria

Laboratory-specific rules defining critical designation.

IN USE apply local critical criteria

notification record

Documentation of the communication and its receipt.

IN USE complete the notification record

sender confirmation

Speaker's check that the repeated message is accurate.

IN USE give sender confirmation

recipient role

Function or authority of the person receiving information.

IN USE record the recipient role

clinical action

Patient-care decision made by an appropriately authorized professional.

IN USE separate clinical action

communication discrepancy

Difference between intended and received information.

IN USE resolve the communication discrepancy

Precision check

Lumen alone supplies the critical designation for this fictional case. Report issue time is 14:10; notification time is 14:12; extension 214 is a contact number. None is a treatment instruction, diagnosis, or universal laboratory threshold.

Phrases you can actually use

C Notice the language / 1 of 2

IDENTIFY THE NOTIFICATION

"This is Lumen laboratory with a critical-result notification for verified case C7."

STATE THE VALUE AND UNIT

"The result is six point four millimoles per liter."

GIVE THE REPORT TIME

"The report was issued at fourteen ten."

INTERRUPT THE ERROR

"Correction: six point four, not six point one."

STATE THE ANALYTE

"The analyte is potassium."

ATTRIBUTE THE DESIGNATION

"Lumen designates this result critical under its own criteria."

REQUEST REPETITION

"Please read back the case, analyte, value, and unit."

REPEAT THE WHOLE VALUE

"Potassium, six point four millimoles per liter."

Correction

Clearly interrupts an inaccurate number rather than politely allowing it to stand.

Not six point one

Identifies the exact mistaken value being replaced.

Complete

Requests analyte and unit as well as the number.

Phrases you can actually use

C Notice the language / 2 of 2

REQUEST A COMPLETE SECOND READ-BACK

"Please repeat the complete message again."

CONFIRM THE RECIPIENT

"You are Dr Shah at extension two one four."

DISTINGUISH TIMES

"The report time is fourteen ten; this call is at fourteen twelve."

SEPARATE CLINICAL DECISIONS

"This notification does not supply a diagnosis or treatment instruction."

CONFIRM ACCURACY

"That read-back is correct."

CONFIRM RECEIPT

"Please confirm receipt of this critical-result notification."

KEEP THE RECORD EXACT

"I will document the recipient, times, correction, and confirmed read-back."

CLOSE THE COMMUNICATION LOOP

"The corrected value and unit have been read back accurately and receipt confirmed."

That ... is correct

Supplies the sender's confirmation after the recipient repeats.

Under its own criteria

Prevents the fictional critical designation becoming a universal threshold.

Report time versus call time

Keeps two distinct events from being combined in the record.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which correction is precise?

- A** Correction: potassium 6.4 mmol/L, not 6.1; please read back the complete result
- B** Yes, close enough
- C** Change it to whatever you heard
- D** It is probably six something

2. Which read-back is complete for the result?

- A** Potassium, six point one
- B** Extension 214 is critical
- C** C7, potassium, 6.4 millimoles per liter
- D** Six point four only

3. What does 214 identify?

- A** A critical threshold
- B** Dr Shah's contact extension
- C** The potassium value
- D** The report time

4. Which conclusion is not authorized by the scenario?

- A** A corrected and confirmed read-back
- B** A specific diagnosis or treatment plan
- C** Lumen's local critical designation
- D** The exact value and unit

Answers and explanations: [page 97](#).

Six point four, not six point one

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

communication discrepancy / read-back / clinical action / closed-loop communication / notification time / complete message / contact extension / numerical value / receipt / millimoles per liter

07

- 01 **Noor:** Dr Shah, this is Noor at Lumen laboratory. For verified case C7, potassium is six point four millimoles per liter, designated critical by Lumen. The report was issued at fourteen ten. Please read it back.
- 02 **Dr Shah:** My ¹ _____ is C7, potassium, six point one millimoles per liter, report issued at fourteen ten. Have I received that correctly?
- 03 **Noor:** Correction: six point four, not six point one. For C7, potassium is six point four millimoles per liter, critical under Lumen's criteria, report issued at fourteen ten. Please repeat the complete message.
- 04 **Dr Shah:** C7, potassium, six point four ² _____, critical under Lumen's criteria, report issued at fourteen ten.
- 05 **Noor:** That read-back is correct. Please confirm receipt of the critical notification. Our case-identity and receiving-authority checks were completed before this exchange.
- 06 **Dr Shah:** I confirm ³ _____ of the critical notification for C7: potassium six point four millimoles per liter, with the report issued at fourteen ten.
- 07 **Noor:** Thank you. This call is at fourteen twelve; the report issue time is fourteen ten. I'll keep those events separate.
- 08 **Dr Shah:** Agreed. Fourteen twelve is the ⁴ _____, not a revised issue time.
- 09 **Noor:** Please confirm extension two-one-four for the communication record. The receiving identity and authority checks are already complete.
- 10 **Dr Shah:** Two-one-four is my ⁵ _____. It isn't part of the concentration or the report time.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 64.

- 11 **Noor:** I'll record that the initial six-point-one repeat was corrected and the complete six-point-four message confirmed.
- 12 **Dr Shah:** Yes, retain that ⁶ _____. The error was in my initial repetition, not a newly amended laboratory value.
- 13 **Noor:** Correct. I haven't changed the result. The report remains potassium six point four millimoles per liter.
- 14 **Dr Shah:** That is the confirmed ⁷ _____. Keep the unit with it, not just the digits.
- 15 **Noor:** The laboratory notification is now acknowledged. No treatment instruction has been supplied in this call.
- 16 **Dr Shah:** Any ⁸ _____ will go through the appropriate patient-care process. Please retain the notification record while I attend to that responsibility.
- 17 **Noor:** I will retain C7, potassium, six point four, millimoles per liter, Lumen's critical designation, and fourteen-ten report time.
- 18 **Dr Shah:** That preserves the ⁹ _____ I received, along with the separate call time and corrected read-back.
- 19 **Noor:** Your accurate repeat and acknowledgment are recorded. I won't substitute message sent for confirmed receipt.
- 20 **Dr Shah:** Thank you. That completes the ¹⁰ _____ for the notification; the subsequent care decisions remain separate.

07

ANSWERS / EXPLANATIONS: PAGE 98

1. read-back | 2. millimoles per liter | 3. receipt | 4. notification time | 5. contact extension | 6. communication discrepancy | 7. numerical value | 8. clinical action | 9. complete message | 10. closed-loop communication

Correct, repeat, confirm

F Say it

Complete the result exchange. Identity and authority are already verified; the critical designation is specific to fictional Lumen.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

millimoles per liter / 14:12 / 14:10 / 6.4

Technician: "Correction: potassium ¹ _____ mmol/L, not 6.1."

Recipient: "The unit means ² _____."

Technician: "The report was issued at ³ _____."

Recipient: "This notification call occurs at ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 98

1. 6.4 | 2. millimoles per liter | 3. 14:10 | 4. 14:12

Rehearse with a fixed script

1. Check the answers, then read turns 1-6 in order. Correct six point one immediately to six point four millimoles per liter and confirm the complete repeat.

2. Switch roles. Read the record details: report fourteen ten, notification fourteen twelve, extension two-one-four. Keep each number attached to its purpose.

3. Complete the four-line exchange with C7, potassium, the correct value and unit. Use only the fictional case; no treatment instruction is supplied.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Delivered parcel, unconfirmed accession, accepted handover

A Read the situation

Distinguish courier delivery from referral acceptance and transfer a pending enquiry with explicit ownership and an update commitment.

Send-out X8 appears on Monday's manifest. The courier confirms parcel delivery on Tuesday, but the referral laboratory has not confirmed accession acceptance for X8. Noor's shift is ending. Incoming technician Eli accepts responsibility for the enquiry and the agreed 15:00 clinic update. No result date is confirmed. The handover must preserve the difference between parcel delivery, individual accession acceptance, and the promise to provide an update rather than a result.

PEOPLE / Noor: Outgoing laboratory technician | Eli: Incoming laboratory technician

08

A MESSAGE LEFT IS NOT A RESPONSIBILITY ACCEPTED

A useful handover ends with the incoming colleague naming the work they accept. Keep the exact unresolved question and the promised update time visible. Evidence of transport progress should not be upgraded to specimen acceptance or a confirmed result date.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does the courier confirm?

A A Friday reporting promise

C X8's accession acceptance

B Parcel delivery on Tuesday

D A completed patient result

2. What remains unconfirmed?

A X8's accession acceptance

C Parcel delivery on Tuesday

B Its appearance on Monday's manifest

D Eli's acceptance of the enquiry

3. What does Eli accept?

A A guaranteed result by 15:00

C Responsibility for an already final result

B Permission to invent a referral number

D The enquiry and the 15:00 clinic update

Answers and explanations: page 99.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

referral laboratory

External laboratory receiving work referred for testing.

IN USE contact the referral laboratory

send-out

Specimen or request sent to another laboratory for work.

IN USE track the send-out

manifest

Record listing items included in a shipment.

IN USE check the manifest

courier

Service transporting the parcel.

IN USE contact the courier

parcel delivery

Arrival of a transport package at its destination.

IN USE confirm parcel delivery

accession acceptance

Referral laboratory's recorded acceptance of the individual request or specimen.

IN USE confirm accession acceptance

delivery confirmation

Evidence that the transport package arrived.

IN USE retain delivery confirmation

referral reference

Identifier used for the externally referred work.

IN USE request the referral reference

tracking number

Transport identifier used to follow a shipment.

IN USE verify the tracking number

chain of custody

Documented sequence of possession and transfer where required.

IN USE preserve chain of custody

dispatch record

Record of sending an item onward.

IN USE consult the dispatch record

outstanding enquiry

Follow-up question not yet resolved.

IN USE transfer the outstanding enquiry

08

Precision check

Monday's manifest records X8 in the outgoing shipment, and Tuesday's courier confirmation establishes parcel delivery. Neither document by itself establishes the referral laboratory's accession acceptance of X8.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

handover

Transfer of status, responsibility, and next actions.

IN USE complete the handover

incoming technician

Staff member taking over work at a shift transition.

IN USE identify the incoming technician

outgoing technician

Staff member transferring work before leaving.

IN USE support the outgoing technician

action owner

Person explicitly responsible for the next task.

IN USE name the action owner

acceptance acknowledgment

Explicit statement accepting a responsibility or received item.

IN USE obtain acceptance acknowledgment

update time

Agreed time to communicate current status.

IN USE preserve the update time

result date

Confirmed or proposed date for an issued finding.

IN USE avoid inventing a result date

pending accession

Status where registration or acceptance remains unresolved.

IN USE report pending accession

transport milestone

Recorded event in the movement of a parcel.

IN USE distinguish a transport milestone

laboratory milestone

Recorded event in laboratory processing or acceptance.

IN USE confirm the laboratory milestone

follow-up ownership

Assigned responsibility for continuing an enquiry.

IN USE transfer follow-up ownership

continuity

Preservation of work and communication across a transition.

IN USE maintain continuity

Precision check

Eli accepts both the unresolved enquiry and the agreed 15:00 clinic update. That creates an explicit owner for follow-up, not a confirmed result date. The update may need to say acceptance is still unconfirmed if that remains the actual status.

Phrases you can actually use

C Notice the language / 1 of 2

IDENTIFY THE SEND-OUT

"This handover concerns X8."

STATE THE TRANSPORT EVIDENCE

"The courier confirms parcel delivery on Tuesday."

AVOID UPGRADING EVIDENCE

"Delivered does not mean individually accepted."

TRANSFER RESPONSIBILITY

"Can you accept this outstanding enquiry?"

STATE THE DISPATCH EVIDENCE

"X8 appears on Monday's manifest."

NAME THE MISSING MILESTONE

"Referral accession acceptance is not confirmed."

NAME THE ENQUIRY

"We need confirmation of X8's accession acceptance."

ACCEPT EXPLICITLY

"I accept responsibility for following up X8."

08

Appears on

Attributes X8's dispatch listing to the manifest.

Confirms delivery

Limits the courier's evidence to transport.

Individually accepted

Distinguishes the specimen's laboratory status from its parcel.

Phrases you can actually use

C Notice the language / 2 of 2

PRESERVE THE CLINIC COMMITMENT

"The agreed clinic update is at fifteen hundred."

AVOID INVENTING A REFERENCE

"Do not create a referral accession number."

REQUEST AN ACCURATE UPDATE

"Report the actual status even if acceptance remains unresolved."

CLOSE THE SHIFT HANDOVER

"Eli has accepted the enquiry and the clinic update."

SEPARATE RESULT TIMING

"No result date is confirmed."

KEEP THE SOURCE VISIBLE

"Attribute delivery to the courier and acceptance to the referral laboratory."

PRESERVE THE RECORD

"Keep the manifest and delivery confirmation linked to X8."

MAINTAIN THE BOUNDARY

"Follow-up accepted does not mean testing completed."

08

I accept responsibility

Makes task ownership explicit.

Even if

Preserves the update obligation when the enquiry is still unresolved.

Does not mean

Prevents one completed milestone from implying another.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which summary is accurate?

- A** The manifest confirms a reporting date
- B** X8 is on Monday's manifest; its parcel arrived Tuesday; accession acceptance is unconfirmed
- C** X8 is accepted because the parcel arrived
- D** X8 has a final result

08

2. What is the precise follow-up question?

- A** Has the referral laboratory accepted X8's accession?
- B** Did the courier diagnose the patient?
- C** Can we invent a result date?
- D** Was every parcel in the country delivered?

3. What should happen at 15:00 under the agreed handover?

- A** A final result is guaranteed
- B** The enquiry disappears automatically
- C** Noor is assumed to remain on shift
- D** Eli gives the clinic the actual status

4. Which statement confirms ownership?

- A** I accept the X8 enquiry and the 15:00 clinic update
- B** Someone probably saw the note
- C** Delivered means everything is finished
- D** No reply means accepted

Answers and explanations: page 99.

Delivered parcel, unconfirmed accession, accepted handover

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

update time / acceptance acknowledgment / result date / outstanding enquiry / follow-up ownership / continuity / referral reference / delivery confirmation / laboratory milestone / dispatch record

08

- 01 **Noor:** Can you take over X8 before I leave? Monday's manifest lists it, and the parcel was delivered Tuesday, but one laboratory status is still missing.
- 02 **Eli:** What is the outstanding ¹ _____? I want to separate what the courier has confirmed from what the referral laboratory has actually acknowledged.
- 03 **Noor:** It's the individual accession acceptance. The referral laboratory hasn't confirmed accepting X8, even though the courier delivered the parcel.
- 04 **Eli:** Then the ² _____ is specifically about X8's accession acceptance. I will not treat parcel arrival as evidence that this particular item was accepted for work.
- 05 **Noor:** I'll leave you both records. The manifest supports dispatch; the courier record supports arrival. Neither is an issued-result notice.
- 06 **Eli:** I will preserve the ³ _____ and the courier evidence. I will not replace those source details with a single vague status such as received and complete.
- 07 **Noor:** The clinic is expecting a status update at fifteen hundred. That commitment remains even if the acceptance enquiry is unresolved.
- 08 **Eli:** I accept the ⁴ _____ of fifteen hundred as part of the handover. If accession acceptance remains unresolved, the update must state that accurately.
- 09 **Noor:** Can you explicitly take the enquiry as well as the call? I need to know who will continue it after this shift.
- 10 **Eli:** Yes. I accept ⁵ _____ for X8's accession enquiry and the clinic update. I will carry both tasks forward under the laboratory's actual process.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 73.

- 11 **Noor:** The referral's accession field is blank in the information I have. X8 is our send-out reference, not a newly confirmed external accession.
- 12 **Eli:** Understood. A ⁶ _____ needs the appropriate source. I will preserve X8 as the send-out reference while seeking the missing acceptance information.
- 13 **Noor:** If someone says received, please ask whether they mean the whole parcel or acceptance of the individual specimen.
- 14 **Eli:** I will call it ⁷ _____, not accession confirmation. The referral laboratory must establish the individual laboratory status we are asking about.
- 15 **Noor:** We also have no result date. Please don't turn the fifteen-hundred update into a promise that the report will be ready.
- 16 **Eli:** No ⁸ _____ is confirmed. I will not turn the fifteen-hundred update into a reporting promise or infer a date from the transport timeline.
- 17 **Noor:** I'll read back the handover: X8 dispatched Monday, parcel delivered Tuesday, accession acceptance outstanding; you own that enquiry and the fifteen-hundred update.
- 18 **Eli:** That is my ⁹ _____ of the handover. I accept those two responsibilities, with the accession status and result timing still unresolved as stated.
- 19 **Noor:** Thanks. I'll record your acceptance before leaving. The unresolved enquiry now has an owner, but the laboratory status itself has not changed.
- 20 **Eli:** That maintains ¹⁰ _____. I will follow up the acceptance question and give the clinic the actual status at fifteen hundred, without claiming a result that has not been confirmed.

08

ANSWERS / EXPLANATIONS: PAGE 100

1. laboratory milestone | 2. outstanding enquiry | 3. dispatch record | 4. update time | 5. follow-up ownership | 6. referral reference | 7. delivery confirmation | 8. result date | 9. acceptance acknowledgment | 10. continuity

Transfer the question and the commitment

F Say it

Complete the shift handover without upgrading parcel delivery to laboratory acceptance.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

15:00 / unconfirmed / X8 / Tuesday

08

Outgoing: "The send-out reference is ¹ _____."

Incoming: "The courier confirms delivery on ² _____."

Outgoing: "Accession acceptance remains ³ _____."

Incoming: "I accept the enquiry and the ⁴ _____ clinic update."

ANSWERS / EXPLANATIONS: PAGE 100

1. X8 | 2. Tuesday | 3. unconfirmed | 4. 15:00

Rehearse with a fixed script

1. Check the dialogue, then read the timeline: Monday dispatch, Tuesday parcel delivery, individual accession acceptance unconfirmed.
2. Switch roles. Have Eli explicitly accept both the X8 enquiry and the fifteen-hundred clinic update, not just receipt of a note.
3. Complete the four-line exchange. Preserve the absent result date and do not turn the update time into a reporting promise.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Three and a half standard deviations, not three and a half percent

E More workplace conversations / 1-10

Explain a control statistic with the correct denominator and a case-specific action rule.

Fictional control M: established mean 100 mg/L, standard deviation (SD) 2 mg/L, observed result 107 mg/L. The target statistics apply to this control and lot. Local rule: absolute z-score of 3 or more triggers a hold and authorized review. The affected reports are already withheld and the lead notified. Use $z = (\text{result} - \text{mean})/\text{SD}$ and coefficient of variation (CV) = $\text{SD}/\text{mean} \times 100\%$. No review outcome or restart permission is supplied.

PEOPLE / Noor: Laboratory technician | Ben: Quality lead

Use each bank entry once, without changing its form. Answers follow turn 20.

09

WORD + PHRASE BANK

inclusive limit / z-score / review outcome / coefficient of variation / trend / standard deviation

- 01 **Noor:** The worksheet says the control is three-point-five percent high. I think it mixes two different calculations. Can we correct that before the review?
- 02 **Ben:** Yes. The result is seven milligrams per liter above the established mean. Dividing that difference by the two-milligram-per-liter ¹ _____ gives three-point-five.
- 03 **Noor:** So the result is three-and-a-half standard deviations above the mean. For percentage difference, the denominator would be the mean of one hundred.
- 04 **Ben:** Exactly. The ² _____ is positive three-point-five, while the percentage above the mean is seven percent. Keep both labels with their numbers.
- 05 **Noor:** Our local hold rule includes an absolute score of exactly three. This result is above that boundary, and the hold is already in place.
- 06 **Ben:** Correct. An ³ _____ includes equality. Don't rewrite the rule as greater than three when describing why it applies.
- 07 **Noor:** What does two divided by one hundred tell us? Someone has added two percent next to the control's historical statistics.
- 08 **Ben:** That is the ⁴ _____ for the supplied statistics. It does not make this particular result acceptable or establish a completed review.
- 09 **Noor:** Would it be accurate to call this a trend? We have only this one observation in the exercise, not a sequence moving upward.
- 10 **Ben:** No. A ⁵ _____ needs a pattern over time. A single point can trigger the stated rule without telling us why it occurred.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 76.

- 11 **Noor:** Then I should not write that calibration caused it, or that recalibration is the solution. Neither is in the evidence we have.
- 12 **Ben:** Right. The ⁶ _____ remains pending. Preserve the result and its context rather than replacing an unexplained observation with a guessed cause.
- 13 **Noor:** On the Levey-Jennings display, I would locate this point above the plus-three-standard-deviation line, provided those are the same applicable target statistics.
- 14 **Ben:** Yes. The stated target and lot match here. A graph can make the distance visible, but the picture does not supply a missing investigation.
- 15 **Noor:** I'll revise the note to result one hundred seven, mean one hundred, SD two, all in milligrams per liter, and z positive three-point-five.
- 16 **Ben:** Include the separate seven-percent difference only if you label its denominator. Otherwise readers may mistake it for the CV or standardized score.
- 17 **Noor:** The established CV is two percent. I will not describe that as this run's newly demonstrated precision from one observation.
- 18 **Ben:** Good. Nor should the instrument's ready indicator be treated as cancellation of the report hold. Those are different kinds of status.
- 19 **Noor:** The handoff will preserve the held reports, notification already made, calculation correction, and pending authorized review.
- 20 **Ben:** That gives the next reviewer a usable record without turning a language exercise into instructions to adjust the analyzer or release results.

09

ANSWERS / EXPLANATIONS: PAGE 101

1. standard deviation | 2. z-score | 3. inclusive limit | 4. coefficient of variation | 5. trend | 6. review outcome

Equality still triggers this local rule

F Say it in a new situation

New fictional control: established mean 200 mg/L, SD 5 mg/L, observed 215 mg/L. Same local rule: absolute z-score of 3 or more triggers a hold and review. The lead is notified; no release is authorized.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

15 / 2.5 / held / three

09

Technician: The result exceeds the mean by ¹ _____ mg/L.

Lead: The z-score is positive ² _____.

Technician: The established CV is ³ _____ percent.

Lead: The affected reports remain ⁴ _____.

ANSWERS / EXPLANATIONS: PAGE 101

1. 15 | 2. three | 3. 2.5 | 4. held

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

The analyzer already applied the dilution factor

E More workplace conversations / 1-10

Reconcile raw and adjusted values without multiplying an already adjusted result twice.

Fictional training record: one sample volume plus four diluent volumes, factor 5. Raw diluted-sample reading: 40 mg/L. Confirmed analyzer flag: factor already applied; adjusted output: 200 mg/L. An unreleased laboratory information system (LIS) draft multiplies by 5 again, showing 1,000 mg/L. This is not a patient report or permission to perform a dilution, alter settings, or release results.

PEOPLE / Noor: Laboratory technician | Imani: Laboratory systems specialist

Use each bank entry once, without changing its form. Answers follow turn 20.

10

WORD + PHRASE BANK

reconciliation record / raw reading / dilution factor / factor-applied flag / unit label / double application

- 01 **Noor:** The instrument output is two hundred milligrams per liter, but the unreleased information-system draft says one thousand. I want to trace where the extra multiplication happened.
- 02 **Imani:** Start with the recorded ¹ _____. One sample volume plus four diluent volumes means a factor of five, not four.
- 03 **Noor:** The raw reading from the diluted material is forty milligrams per liter. Multiplying forty by five gives two hundred for the original-sample equivalent.
- 04 **Imani:** Correct. Keep the ² _____ separate from the adjusted output in the comparison. Both happen to use the same concentration unit.
- 05 **Noor:** The documented analyzer flag confirms that it already applied five. That is stronger evidence than guessing from the size of the displayed number.
- 06 **Imani:** Yes. The ³ _____ explains what the output represents. Instrument and interface behavior must be checked, not assumed identical across systems.
- 07 **Noor:** Then the draft's second multiplication is the source of one thousand. It is five times the correctly adjusted output, not a different dilution.
- 08 **Imani:** That is ⁴ _____ of the same factor. We need to preserve that distinction in the discrepancy record.
- 09 **Noor:** Could someone describe the difference as a units conversion? Forty, two hundred, and one thousand are all labeled milligrams per liter here.
- 10 **Imani:** No. The ⁵ _____ has not changed. Don't attach a conversion explanation to an arithmetic step that applied a dilution factor again.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 79.

- 11 **Noor:** I can identify the mismatch, but that does not mean I should overwrite the draft or change the interface setting during this discussion.
- 12 **Imani:** Exactly. The ⁶ _____ should retain the evidence and go through the authorized review and correction process.
- 13 **Noor:** I'll list raw forty, factor five, adjusted two hundred, draft one thousand, with the same unit alongside each concentration.
- 14 **Imani:** Also identify which field supplied each value. A screenshot of a number alone can lose the distinction between raw and adjusted fields.
- 15 **Noor:** We should keep the original discrepant draft traceable if an authorized correction is made. A clean final value would not explain what changed.
- 16 **Imani:** Agreed. Preserve the applicable change history. Do not remove the evidence needed to understand the repeated multiplication.
- 17 **Noor:** And we should not claim this proves every connected analyzer handles dilution the same way. This flag was verified for this record.
- 18 **Imani:** Right. The historical manufacturer example supports why system-specific handling matters, not a universal setting or current operating procedure.
- 19 **Noor:** My handoff will say the arithmetic is reconciled to two hundred, the draft differs by a repeated factor, and release remains unauthorized.
- 20 **Imani:** That separates a checked calculation from permission to issue a report. The responsible reviewer still needs to resolve the record through the actual laboratory process.

10

ANSWERS / EXPLANATIONS: PAGE 102

1. dilution factor | 2. raw reading | 3. factor-applied flag | 4. double application | 5. unit label | 6. reconciliation record

Identify the second multiplication

F Say it in a new situation

New fictional record: one sample volume plus nine diluent volumes; raw reading 7 mg/L. The confirmed analyzer flag says the factor was already applied, and output is 70 mg/L. An unreleased draft shows 700 mg/L after multiplying again. Authorized review is not complete.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

10 / 700 / 70 / pending

10

Technician: The recorded dilution factor is ¹ _____.

Specialist: The once-adjusted concentration is ² _____ mg/L.

Technician: The twice-adjusted draft incorrectly shows ³ _____ mg/L.

Specialist: Authorized review is still ⁴ _____.

ANSWERS / EXPLANATIONS: PAGE 102

1. 10 | 2. 70 | 3. 700 | 4. pending

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

Detected does not mean reliably quantified

E More workplace conversations / 1-10

Preserve analytical qualifiers without converting detection into a diagnosis or precise concentration.

Fictional assay M: documented detection limit 0.20 mg/L and quantification limit 0.60 mg/L. A validated positive detection call is confirmed; the internal concentration estimate is 0.35 mg/L. This laboratory's stated reporting rule requires 'detected, below quantification limit' for this case, without publishing the estimate as a numerical result. Clinical decision limits are not supplied. The report remains unreleased.

PEOPLE / Noor: Laboratory technician | Leila: Reporting reviewer

Use each bank entry once, without changing its form. Answers follow turn 20.

11

WORD + PHRASE BANK

internal estimate / Display resolution / clinical decision limit / detection limit / reporting qualifier / quantification limit

- 01 **Noor:** The screen displays zero-point-three-five milligrams per liter, but the draft report uses a qualifier. Are we omitting a result that the instrument measured?
- 02 **Leila:** The ¹ _____ is below the stated quantification limit. The display alone doesn't establish that we may publish that number as a reliable concentration.
- 03 **Noor:** Detection is already confirmed through the validated procedure. I should not infer it merely because the displayed estimate is above zero-point-two-zero.
- 04 **Leila:** Correct. The ² _____ and the confirmed detection call provide different information. Use the stated call, not an invented interpretation of a screen.
- 05 **Noor:** The quantification boundary is zero-point-six-zero. So detection can be confirmed even though this case does not meet the requirement for a numerical report.
- 06 **Leila:** Yes. The ³ _____ concerns suitable quantitative performance. It is not another name for the detection limit.
- 07 **Noor:** Would writing zero be clearer for the recipient? It would avoid presenting the uncertain zero-point-three-five value.
- 08 **Leila:** No. The required ⁴ _____ is detected, below quantification limit. Zero would contradict the confirmed detection and erase important information.
- 09 **Noor:** Then I also shouldn't say below the detection limit. That is a different boundary and would confuse why this qualifier is being used.
- 10 **Leila:** Exactly. Nor is an analytical limit a ⁵ _____. We have no supplied basis for calling this evidence of a disease or a normal patient finding.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 82.

- 11 **Noor:** The two decimal places on the screen look very precise. But the number of printed digits does not establish measurement precision at that concentration.
- 12 **Leila:** Right. ⁶ _____ and demonstrated analytical performance are not the same thing. Keep that distinction when someone asks why the displayed number is not the report.
- 13 **Noor:** Should the report simply say less than zero-point-six? That would retain the upper quantitative boundary but leave out the confirmed detection.
- 14 **Leila:** Use the complete required wording for this case. A bare less-than statement would drop information that the supplied rule says to retain.
- 15 **Noor:** I'll keep the internal estimate in the appropriate traceable record, not move it into a patient-facing numerical field on my own.
- 16 **Leila:** Yes. Follow the actual reporting procedure and authorized review. This language exercise doesn't grant permission to change report fields.
- 17 **Noor:** The background definitions distinguish detection from quantification. They do not supply this fictional laboratory's reporting policy or a clinical decision threshold.
- 18 **Leila:** Correct. The provided policy is part of the case. Do not attribute it to the pharmaceutical analytical-validation reference as a universal clinical reporting instruction.
- 19 **Noor:** My handoff will retain confirmed detection, the below-quantification qualifier, and unreleased status, with no diagnosis or precise published concentration.
- 20 **Leila:** That gives the next reviewer the important distinctions. A cautious message should preserve what is known, not replace it with either false certainty or a false zero.

11

ANSWERS / EXPLANATIONS: PAGE 103

1. internal estimate | 2. detection limit | 3. quantification limit | 4. reporting qualifier | 5. clinical decision limit | 6. Display resolution

Retain the positive detection and its qualifier

F Say it in a new situation

New fictional assay: detection limit 0.10 mg/L, quantification limit 0.40 mg/L, internal estimate 0.25 mg/L. Detection is confirmed by the validated procedure. The stated local rule requires detected, below quantification limit; the report is unreleased. No clinical interpretation is supplied.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

quantification / 0.40 / detected / unsupplied

11

Technician: The confirmed analytical call is ¹ _____.

Reviewer: The quantification limit is ² _____ mg/L.

Technician: The required qualifier is below the ³ _____ limit.

Reviewer: Clinical significance remains ⁴ _____.

ANSWERS / EXPLANATIONS: PAGE 103

1. detected | 2. 0.40 | 3. quantification | 4. unsupplied

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

01 Briefing and language checks

Accessioning and unambiguous identifiers

A Briefing checks / page 4

1. Which accession appears in the receiving log?

B. A314

The briefing identifies A314 as the accession in the receiving log.

2. Which reference does the caller request?

C. A341

Mira asks about A341, which differs from the logged A314.

3. What has been established about identity?

A. No match has been established

Similar digits do not establish a match, an error, or permission to disclose.

D Language checks / page 9

1. Which response is appropriate before verification?

B. You asked about A341; please complete the required case verification

The appropriate response repeats the supplied reference and preserves the verification requirement.

2. What does the digit resemblance establish?

B. A possible mismatch requiring checking, not identity

Similar digits do not establish identity, responsibility for an error, or report status.

3. Which action preserves traceability?

A. Keep the supplied reference and document the unresolved enquiry

Retaining the actual request prevents a guessed correction from obscuring the enquiry.

4. Which final status is accurate?

B. Identity verification remains required

The scenario ends without an established case match or broader search conclusion.

01 Conversation answers

Accessioning and unambiguous identifiers

E Dialogue gaps

Pages 10-11

1. verification. Turn 2. Verification must establish the correct case and permitted recipient before Noor discusses case-specific information.

2. transcription error. Turn 4. Transcription error is only a possibility, not an established explanation for the difference between references.

3. accession log. Turn 6. Accession log is the laboratory record Noor can check, without treating a similar entry as an authorized case match.

4. case match. Turn 8. Case match is the verified link between the clinic's enquiry and the correct laboratory record, which remains unestablished.

5. secure channel. Turn 10. Secure channel is the approved protected route for the required verification information rather than unrestricted disclosure in the enquiry.

6. source record. Turn 12. Source record provides the authoritative reference to check rather than relying on a remembered or guessed number.

7. Receiving authority. Turn 14. Receiving authority concerns permission to receive the specific information and is separate from merely claiming a clinic affiliation.

8. unverified reference. Turn 16. Unverified reference accurately describes A341 while the enquiry has not been linked to the correct case through the required checks.

9. audit trail. Turn 18. Audit trail preserves the supplied reference, subsequent correction, and verification actions instead of silently replacing the original request.

10. Confidentiality. Turn 20. Confidentiality requires protecting case information while the correct identity and recipient authority remain unresolved.

F Transfer conversation

Page 12

1. A314

Technician: "The receiving log lists A314."

A314 is the reference explicitly supplied for the receiving log.

2. A341

Caller: "The requested reference is A341."

Mira supplied A341, which differs from the logged accession.

3. unverified

Technician: "The case match remains unverified."

No required verification has established that the two references identify the same case.

4. process

Caller: "I will follow the approved verification process."

The requested case must be identified through the laboratory's applicable arrangements.

02 Briefing and language checks

Specimen suitability and recollection language

A Briefing checks / page 13

1. What does the lead document for Q6?

A. Insufficient specimen quantity

The documented finding concerns the amount available for the requested test.

2. What action has the lead requested?

B. Recollection through the clinic

The scenario explicitly supplies a recollection request through the clinic.

3. Which detail is not supplied?

C. Collection method

No collection method, urgency, or cause is included in the stated facts.

D Language checks / page 18

1. Which explanation is accurate?

B. There is insufficient specimen quantity for Q6, not a low patient result

The message concerns suitability of the available specimen amount only.

2. Which action is documented?

C. Recollection through the clinic

Only recollection through the clinic is stated as the lead's requested action.

3. Which statement wrongly adds a cause?

C. The collector definitely used the wrong container

The scenario supplies no cause, so blaming a container or collector is unsupported.

4. How should urgency be described?

D. Not supplied in this message; follow the applicable escalation arrangements

No urgency classification is given, so neither routine nor critical may be invented.

02 Conversation answers

Specimen suitability and recollection language

E Dialogue gaps

Pages 19-20

- 1. specimen volume.** Turn 2. Specimen volume is the amount available for testing, not the patient's measured test result.
- 2. test request.** Turn 4. Test request Q6 identifies the work associated with the documented quantity issue and recollection request.
- 3. patient result.** Turn 6. Patient result has not been supplied, so Noor must not invent a value or a low-result interpretation.
- 4. recollection.** Turn 8. Recollection is the lead's requested next action through the clinic, without a supplied method or urgency classification.
- 5. rejection reason.** Turn 10. Rejection reason here is insufficient quantity for Q6, which does not establish who caused it or how it happened.
- 6. preanalytical phase.** Turn 12. Preanalytical phase concerns work before measurement, and this specimen issue must not be recast as an analytical patient finding.
- 7. authorized guidance.** Turn 14. Authorized guidance is needed for missing urgency or collection details rather than inventing a classification from the quantity message.
- 8. specimen requirement.** Turn 16. Specimen requirement must be established from the relevant test instructions, not inferred into a collection or reporting promise.
- 9. recollection request.** Turn 18. Recollection request is the documented action to obtain another specimen through the clinic without additional invented details.
- 10. clinical interpretation.** Turn 20. Clinical interpretation concerns patient meaning and is not provided by this specimen-quantity communication.

F Transfer conversation

Page 21

1. Q6

Technician: "The request reference is Q6."

The documented insufficient-quantity message concerns test request Q6.

2. quantity

Clinic: "The issue is insufficient specimen quantity."

The finding concerns the amount available for testing, not a patient concentration.

3. recollection

Technician: "The lead requested recollection through the clinic."

Recollection through the clinic is the only supplied next action.

4. result

Clinic: "This is not a low patient result."

No patient result or low measured concentration is supplied in this message.

03 Briefing and language checks

Quality-control status and reportable results

03

A Briefing checks / page 22

1. Which stage is complete?

C. The analytical run

The instrument has finished the analytical run, but review and release remain unresolved.

2. What is the quality-control status?

A. Review unresolved

No resolved quality-control finding is supplied, so neither pass nor failure may be assumed.

3. Which release claim is supported?

B. The lead has not authorized release

The run is complete, but only the stated laboratory process can authorize release; no such permission is supplied.

D Language checks / page 27

1. Which update is accurate?

A. The run is complete, QC review is unresolved, and release is not authorized

The correct update preserves all three distinct process and authorization states.

2. What does unresolved QC review establish?

A. No completed review outcome is supplied

An unresolved review supports neither a pass nor a failure conclusion.

3. Who should address the review question?

B. The laboratory lead under the authorized process

The stated release and review authority belongs to the laboratory process and lead.

4. Which action misrepresents status?

A. Calling the results final without authorization

Final status cannot be created merely by changing the wording of an unreleased result.

03 Conversation answers

Quality-control status and reportable results

03

E Dialogue gaps

Pages 28-29

1. analytical run. Turn 2. Analytical run completion confirms that the measurement sequence finished, not that review or release has been authorized.

2. pending review. Turn 4. Pending review accurately states the unresolved quality assessment without suggesting it has passed or failed.

3. review outcome. Turn 6. Review outcome is not supplied, so an unresolved assessment cannot be reported as a confirmed pass or failure.

4. status update. Turn 8. Status update communicates the current workflow stage without inventing patient findings or an analytical conclusion.

5. release authorization. Turn 10. Release authorization must come through the applicable laboratory process, not from the clinic's request for final wording.

6. authorized reviewer. Turn 12. Authorized reviewer is the role that must resolve the quality assessment under the laboratory process before a release claim is made.

7. report status. Turn 14. Report status remains unreleased and not final, even though the instrument's analytical stage is complete.

8. reportable result. Turn 16. Reportable result must meet the relevant reporting requirements, which are not established by instrument completion alone.

9. quality control. Turn 18. Quality control is the analytical-performance review area still unresolved, not an optional step erased by the completed run.

10. withheld result. Turn 20. Withheld result describes information not released pending the applicable review and authorization rather than a final report.

F Transfer conversation

Page 30

1. complete

Technician: "The analytical run is complete."

The instrument has finished the stated analytical sequence.

2. unresolved

Clinic: "Quality-control review remains unresolved."

No completed quality-control review outcome is supplied in this scenario.

3. authorized

Technician: "Result release is not authorized."

The laboratory lead has not given permission to release results.

4. final

Clinic: "We must not call the report final."

Final reporting is not established by the instrument's completed run.

04 Briefing and language checks

Reagents, instruments, and service coordination

04

A Briefing checks / page 31

1. What is H2's current recorded status?

A. Unavailable

H2 is explicitly marked unavailable, with no confirmed return to service.

2. What happened at 11:00?

A. The service visit ended

Eleven o'clock is only the recorded end of the service visit.

3. What does Noor need before the clinic update?

A. The lead's confirmed status

Noor needs the laboratory lead's documented status rather than an inference from the visit.

D Language checks / page 36

1. Which clinic update is supported?

A. H2 remains unavailable; the service visit ended at 11:00 but return is unconfirmed

The supplied facts distinguish completed service attendance from unresolved operational status.

2. What would establish a status change here?

A. The laboratory lead's confirmed return-to-service status

The lead has not yet confirmed return, so that explicit status is needed.

3. Which detail must not be invented?

C. A restart time

No restart time is supplied by the known service event.

4. What does the service visit prove?

B. Support personnel attended and the visit ended

Attendance and its end are the only service facts established in the scenario.



04 Conversation answers

Reagents, instruments, and service coordination

E Dialogue gaps

Pages 37-38

- 1. instrument status.** Turn 2. Instrument status remains unavailable until the lead confirms a documented change, regardless of the service visit ending.
- 2. service visit.** Turn 4. Service visit records support attendance and its completion, not automatic approval that the instrument is operational.
- 3. Return to service.** Turn 6. Return to service is the authorized restoration of operational use and has not been confirmed for H2.
- 4. service report.** Turn 8. Service report can document service work or findings, but its contents and resulting authorization must be reviewed rather than assumed.
- 5. reagent lot.** Turn 10. Reagent lot identifies a production batch and is not a documented cause of H2's unavailability in this scenario.
- 6. Calibration.** Turn 12. Calibration is a technical measurement-related process whose successful completion is not established by the service visit ending.
- 7. downtime.** Turn 14. Downtime is the period of unavailability, which remains ongoing in the supplied status despite the completed visit.
- 8. status confirmation.** Turn 16. Status confirmation explicitly establishes the operating state needed for a clinic update rather than relying on a vague service-complete message.
- 9. equipment identifier.** Turn 18. Equipment identifier H2 keeps the status attached to the correct analyzer and prevents confusion with another instrument.
- 10. Operational readiness.** Turn 20. Operational readiness requires the relevant confirmed requirements for use and has not been established for H2.

F Transfer conversation

Page 39

- 1. H2**
Technician: "The analyzer identifier is H2."
H2 is the equipment reference supplied for this service enquiry.
- 2. unavailable**
Lead: "Its recorded status remains unavailable."
No confirmed return-to-service decision has changed the unavailable status.
- 3. 11:00**
Technician: "The service visit ended at 11:00."
Eleven o'clock is the documented end of the service visit only.
- 4. confirmed**
Lead: "Return to service is not yet confirmed."
The laboratory lead has not confirmed operational return for H2.

04

05 Briefing and language checks

Turnaround times and realistic updates

A Briefing checks / page 40

1. When does the quoted turnaround begin?

D. Laboratory receipt

The service explicitly measures the quoted interval from laboratory receipt to reporting.

2. When was T5 received?

D. 09:00

The specimen was collected at eight but received by the laboratory at nine.

3. What report time is confirmed?

A. None

The backlog affects estimates and the scenario supplies no confirmed report time.

D Language checks / page 45

1. Why is the assumed 10:00 time unsupported?

D. It starts the two-hour interval at collection rather than receipt

The clinic uses the wrong starting event for the stated receipt-to-report interval.

2. What does 09:00 plus two hours establish?

B. An 11:00 reference calculation, not a confirmed report time

Arithmetic applies the quoted interval correctly but does not remove the stated uncertainty.

3. Which update is accurate?

A. T5 arrived at nine; backlog affects estimates; no report time is confirmed

The correct update uses the receipt time and preserves the unresolved estimate.

4. What is distinct from a result commitment?

C. An agreed time to provide a status update

An update can communicate progress or continued delay without promising completed results.

05 Conversation answers

Turnaround times and realistic updates

E Dialogue gaps

Pages 46-47

- 1. turnaround time.** Turn 2. Turnaround time for this service is measured from laboratory receipt to reporting, not from specimen collection.
- 2. receipt time.** Turn 4. Receipt time is 09:00 for T5 and is the starting event specified in this service's quoted interval.
- 3. reference calculation.** Turn 6. Reference calculation applies the quoted interval to receipt but does not establish a confirmed report time during the backlog.
- 4. quoted interval.** Turn 8. Quoted interval is the service's stated receipt-to-report duration, which is distinct from a guaranteed time for this specimen.
- 5. expectation reset.** Turn 10. Expectation reset corrects both the collection-based assumption and the unsupported reporting guarantee in the earlier message.
- 6. backlog.** Turn 12. Backlog is explicitly supplied as affecting estimates, while the exact report time remains unconfirmed.
- 7. update commitment.** Turn 14. Update commitment promises communication of status, which may still be pending, rather than guaranteeing that a report is ready.
- 8. escalation route.** Turn 16. Escalation route directs clinical urgency concerns to the authorized process rather than inferring their significance from turnaround arithmetic.
- 9. time reference.** Turn 18. Time reference links each clock time to its actual event so collection and receipt are not confused again.
- 10. confirmed deadline.** Turn 20. Confirmed deadline would require an explicit established reporting commitment, which this scenario does not provide.

F Transfer conversation

Page 48

1. 08:00

Clinic: "T5 was collected at 08:00."

Eight o'clock is the specimen collection time, not the service's starting event.

2. 09:00

Technician: "Laboratory receipt was at 09:00."

Nine o'clock is the recorded receipt time used by the quoted service interval.

3. 11:00

Clinic: "Adding the quoted two hours gives 11:00 as a reference calculation."

Nine plus two hours is eleven, but this arithmetic is not a guarantee.

4. unconfirmed

Technician: "The actual report time remains unconfirmed."

The backlog affects estimates and no reporting commitment is supplied.

05

06 Briefing and language checks

Preliminary, final, and amended reports

A Briefing checks / page 49

1. What was received at 10:00?

A. A preliminary report

The earlier report is explicitly preliminary, despite Noor's mistaken description.

2. Which version supersedes it?

A. The 14:00 final report

The supplied final report at fourteen hundred supersedes the preliminary version for this request.

3. What has not been supplied?

C. Findings or a later amended report

The scenario provides report stages and times but no findings or later amendment.

D Language checks / page 54

1. Which correction is clear?

B. I incorrectly called the 10:00 report final; it was preliminary; the 14:00 report is final

The correct message identifies the error and gives both accurate version statuses.

2. What does supersedes mean here?

D. The later final version replaces the preliminary version for this request

Superseding describes which version is applicable, not deletion or a clinical conclusion.

3. Which claim is unsupported?

B. An amended report was issued after 14:00

No later amendment appears in the supplied report sequence.

4. What should be preserved?

C. A traceable correction and report history under policy

A documented correction should repair the message without concealing the relevant history.

06 Conversation answers

Preliminary, final, and amended reports

E Dialogue gaps

Pages 55-56

- 1. explicit correction.** Turn 2. Explicit correction directly identifies the earlier inaccurate final label instead of leaving the recipient to infer what changed.
- 2. status label.** Turn 4. Status label for the earlier report must be preliminary, correcting Noor's mistaken description without inventing clinical content.
- 3. communication correction.** Turn 6. Communication correction repairs Noor's inaccurate message and does not itself establish an amended clinical report.
- 4. amended report.** Turn 8. Amended report would be a formally revised report under the applicable process, which the stated preliminary-to-final sequence does not establish.
- 5. issue time.** Turn 10. Issue time distinguishes the 10:00 preliminary report from the 14:00 final report in the correction and handoff.
- 6. finding.** Turn 12. Finding refers to report content, which has not been supplied and must not be inferred from a change in report status.
- 7. correction trail.** Turn 14. Correction trail links the inaccurate message to its explicit replacement so the earlier error remains traceable.
- 8. document history.** Turn 16. Document history retains the version sequence and correction record according to policy instead of hiding the earlier report or error.
- 9. receipt.** Turn 18. Receipt establishes that Priya received the corrected version information rather than leaving communication assumed.
- 10. current report.** Turn 20. Current report is the supplied 14:00 final version applicable to this request, with no later amendment established.

F Transfer conversation

Page 57

1. preliminary

Technician: "The 10:00 report was preliminary."

The earlier report's correct status is preliminary despite the mistaken phone label.

2. 14:00

Clinic: "The 14:00 report is final."

Fourteen hundred is the stated time of the later final report.

3. supersedes

Technician: "The final report supersedes the earlier version."

The later final version replaces the preliminary version for this request.

4. amended

Clinic: "No later amended report is supplied."

The scenario establishes no subsequent formally amended report.

06

07 Briefing and language checks

Critical notifications and acknowledged handoffs

A Briefing checks / page 58

1. What potassium value must be communicated?

C. 6.4 mmol/L

Lumen's supplied report value is 6.4 millimoles per liter, not the initial mistaken repeat.

2. What is already verified?

D. Case identity and Dr Shah's receiving authority

The briefing explicitly establishes identity and recipient authority before the call.

3. Which time is the call time?

D. 14:12

The report was issued at 14:10, while the notification call occurs at 14:12.

D Language checks / page 63

1. Which correction is precise?

A. Correction: potassium 6.4 mmol/L, not 6.1; please read back the complete result

The correct response replaces the wrong value and requests complete confirmation.

2. Which read-back is complete for the result?

C. C7, potassium, 6.4 millimoles per liter

The complete result repeat includes the verified case reference, analyte, exact value, and unit.

3. What does 214 identify?

B. Dr Shah's contact extension

Extension 214 is the supplied recipient contact number, not a laboratory measurement.

4. Which conclusion is not authorized by the scenario?

B. A specific diagnosis or treatment plan

The scenario explicitly provides no diagnosis, treatment instruction, or universal threshold.

07 Conversation answers

Critical notifications and acknowledged handoffs

E Dialogue gaps

Pages 64-65

- 1. read-back.** Turn 2. Read-back is the recipient's repetition, which initially contains the incorrect 6.1 value and must be corrected.
- 2. millimoles per liter.** Turn 4. Millimoles per liter is the required concentration unit accompanying the corrected potassium value of 6.4.
- 3. receipt.** Turn 6. Receipt confirms that the verified authorized recipient has received the corrected critical-result message.
- 4. notification time.** Turn 8. Notification time is 14:12 for this call and must remain distinct from the report's 14:10 issue time.
- 5. contact extension.** Turn 10. Contact extension 214 identifies Dr Shah's telephone contact and is not a result value or timestamp.
- 6. communication discrepancy.** Turn 12. Communication discrepancy is the initial 6.1 repeat that was explicitly corrected to 6.4 and confirmed.
- 7. numerical value.** Turn 14. Numerical value is 6.4, while the correction concerns the misheard repeat rather than a changed laboratory report.
- 8. clinical action.** Turn 16. Clinical action belongs to the appropriate authorized patient-care process and is not supplied by this language-practice notification.
- 9. complete message.** Turn 18. Complete message retains the case reference, potassium, corrected 6.4 value, unit, critical designation, and report time.
- 10. closed-loop communication.** Turn 20. Closed-loop communication is completed through the corrected recipient repetition, sender confirmation, and explicit acknowledgment of receipt.

F Transfer conversation

Page 66

1. 6.4

Technician: "Correction: potassium 6.4 mmol/L, not 6.1."

The supplied potassium value is 6.4, correcting the inaccurate initial repeat.

2. millimoles per liter

Recipient: "The unit means millimoles per liter."

The abbreviation mmol/L expands to millimoles per liter.

3. 14:10

Technician: "The report was issued at 14:10."

Fourteen ten is the report issue time, distinct from the later call.

4. 14:12

Recipient: "This notification call occurs at 14:12."

Fourteen twelve is the supplied time of the critical-result notification call.

08 Briefing and language checks

Referral laboratories and outstanding work

A Briefing checks / page 67

1. What does the courier confirm?

B. Parcel delivery on Tuesday

The courier confirms transport delivery, not the referral laboratory's individual accession decision.

2. What remains unconfirmed?

A. X8's accession acceptance

The referral laboratory has not confirmed accession acceptance for X8.

3. What does Eli accept?

D. The enquiry and the 15:00 clinic update

Eli explicitly accepts follow-up ownership and the agreed update, not a result deadline.

D Language checks / page 72

1. Which summary is accurate?

B. X8 is on Monday's manifest; its parcel arrived Tuesday; accession acceptance is unconfirmed

The accurate summary keeps dispatch, delivery, and accession acceptance separate.

2. What is the precise follow-up question?

A. Has the referral laboratory accepted X8's accession?

The outstanding laboratory milestone is X8's individual accession acceptance.

3. What should happen at 15:00 under the agreed handover?

D. Eli gives the clinic the actual status

The commitment concerns a clinic update, not guaranteed testing completion.

4. Which statement confirms ownership?

A. I accept the X8 enquiry and the 15:00 clinic update

Eli's explicit acceptance transfers both the enquiry and the stated communication responsibility.

08 Conversation answers

Referral laboratories and outstanding work

E Dialogue gaps

Pages 73-74

1. laboratory milestone. Turn 2. Laboratory milestone is X8's individual accession acceptance, which is distinct from the confirmed transport delivery.

2. outstanding enquiry. Turn 4. Outstanding enquiry asks whether the referral laboratory has accepted X8, rather than whether its parcel reached the destination.

3. dispatch record. Turn 6. Dispatch record is the Monday manifest listing X8, which supports shipment history but not later acceptance or reporting.

4. update time. Turn 8. Update time is the agreed 15:00 clinic communication commitment, not a confirmed result deadline.

5. follow-up ownership. Turn 10. Follow-up ownership makes Eli responsible for continuing the X8 accession enquiry rather than merely receiving a note.

6. referral reference. Turn 12. Referral reference must come from the relevant confirmed record and must not be invented to make the handover appear complete.

7. delivery confirmation. Turn 14. Delivery confirmation establishes the parcel's arrival on Tuesday but not the referral laboratory's acceptance of every individual item.

8. result date. Turn 16. Result date remains unconfirmed and cannot be inferred from dispatch, parcel arrival, or acceptance of follow-up responsibility.

9. acceptance acknowledgment. Turn 18. Acceptance acknowledgment explicitly confirms Eli's responsibility for both the unresolved enquiry and the agreed clinic update.

10. continuity. Turn 20. Continuity preserves the unresolved work, evidence, and communication commitment across Noor's shift ending and Eli taking over.

F Transfer conversation

Page 75

1. X8

Outgoing: "The send-out reference is X8."

X8 identifies the outstanding referral enquiry in the supplied handover.

2. Tuesday

Incoming: "The courier confirms delivery on Tuesday."

Tuesday is the confirmed parcel-delivery day, not an accession-acceptance confirmation.

3. unconfirmed

Outgoing: "Accession acceptance remains unconfirmed."

The referral laboratory has not confirmed acceptance for the individual send-out.

4. 15:00

Incoming: "I accept the enquiry and the 15:00 clinic update."

Eli accepts responsibility for the agreed fifteen-hundred status update, not a result deadline.

08

09 Conversation answers

Three and a half standard deviations, not three and a half percent

E Extended conversation

Pages 76-77

1. standard deviation. Turn 2. Standard deviation is the supplied spread statistic of 2 mg/L; dividing the 7 mg/L difference by it gives 3.5, not 3.5 percent.

2. z-score. Turn 4. The z-score is $(107 - 100)/2 = +3.5$, a unitless standardized distance above the established mean; the percentage difference is separately seven percent.

3. inclusive limit. Turn 6. The supplied local rule includes an absolute z-score equal to three, as well as values greater than three; it is not a universal laboratory rule.

4. coefficient of variation. Turn 8. The supplied established SD divided by its mean, multiplied by one hundred, is two percent; this CV describes those established statistics, not proof that the current run is acceptable.

5. trend. Turn 10. A trend requires a pattern across observations; the single supplied high point cannot establish a sustained directional pattern or its cause.

6. review outcome. Turn 12. No authorized review outcome is supplied; the hold and correct arithmetic do not establish a cause, technical remedy, or permission to release the affected reports.

F Transfer exchange

Page 78

1. 15

Technician: The result exceeds the mean by 15 mg/L.

Subtract the established mean of two hundred from the observed result of two hundred fifteen.

2. three

Lead: The z-score is positive three .

The difference of fifteen divided by the SD of five equals positive three.

3. 2.5

Technician: The established CV is 2.5 percent.

Five divided by two hundred, multiplied by one hundred, equals two-point-five percent.

4. held

Lead: The affected reports remain held .

The case-specific rule includes equality at an absolute z-score of three, and no release is authorized.

Background reading: [WHO laboratory quality training, module 7: SD, CV and control-chart terminology](#); local rule is fictional. The conversation is an original fictional teaching case.

10 Conversation answers

The analyzer already applied the dilution factor

E Extended conversation

Pages 79-80

1. dilution factor. Turn 2. One volume of sample plus four volumes of diluent gives five final volumes per one sample volume, so the recorded dilution factor is five, not four.

2. raw reading. Turn 4. The raw diluted-sample reading is forty mg/L before the factor is applied; it must remain distinguishable from the adjusted two-hundred-mg/L output.

3. factor-applied flag. Turn 6. The setup explicitly confirms the analyzer's documented flag, so its output of two hundred mg/L already includes the factor and must not be treated as an unadjusted reading.

4. double application. Turn 8. Multiplying the already adjusted two hundred by five produces one thousand, applying the same dilution factor twice rather than adding new measurement information.

5. unit label. Turn 10. All three supplied values use mg/L, so the discrepancy is not an mg/L-to-another-unit conversion; the extra multiplication is the documented difference.

6. reconciliation record. Turn 12. The reconciliation record should retain the raw value, factor, confirmed flag, adjusted output and discrepant draft for authorized review; identifying the mismatch does not itself authorize edits or release.

F Transfer exchange

Page 81

1. 10

Technician: The recorded dilution factor is 10 .

One sample volume plus nine diluent volumes gives ten final volumes per sample volume.

2. 70

Specialist: The once-adjusted concentration is 70 mg/L.

Seven multiplied by the factor of ten equals seventy, which is the supplied adjusted analyzer output.

3. 700

Technician: The twice-adjusted draft incorrectly shows 700 mg/L.

Multiplying the already adjusted seventy by ten again produces the discrepant seven hundred.

4. pending

Specialist: Authorized review is still pending .

The calculation can be checked, but the setup states that authorized review has not been completed.

10

Background reading: Beckman Coulter REMISOL Advance v1.7, historical section 3.14.4: instrument-specific dilution handling, not current operating instructions. The conversation is an original fictional teaching case.

11 Conversation answers

Detected does not mean reliably quantified

E Extended conversation

Pages 82-83

1. internal estimate. Turn 2. The internal estimate is 0.35 mg/L, below the stated 0.60 mg/L quantification limit; its display does not make it a reportable precise numerical concentration under this case's rule.

2. detection limit. Turn 4. The stated detection limit is 0.20 mg/L; the case separately supplies a confirmed detection call, rather than authorizing a positive call from a simple numerical comparison alone.

3. quantification limit. Turn 6. The quantification limit concerns measuring an amount with suitable precision and accuracy; the case gives 0.60 mg/L and requires a qualifier below it, rather than a numerical report of 0.35.

4. reporting qualifier. Turn 8. The case's reporting rule requires detected, below quantification limit; replacing it with zero or not detected would contradict the confirmed detection call.

5. clinical decision limit. Turn 10. Clinical decision limits concern clinical interpretation and are not supplied here; analytical detection or quantification limits must not be presented as diagnostic or normal-range cutoffs.

6. Display resolution. Turn 12. Display resolution describes how finely a screen presents values; two decimal places do not establish reliable quantification, measurement precision, or clinical significance.

F Transfer exchange

Page 84

1. detected

Technician: The confirmed analytical call is detected .

The setup explicitly confirms detection; neither zero nor not detected preserves that finding.

2. 0.40

Reviewer: The quantification limit is 0.40 mg/L.

The case supplies a quantification limit of zero-point-four-zero, distinct from the detection limit of zero-point-one-zero.

3. quantification

Technician: The required qualifier is below the quantification limit.

The internal estimate lies below the supplied quantification limit, and the case explicitly requires that qualifier.

4. unsupplied

Reviewer: Clinical significance remains unsupplied .

No clinical interpretation or clinical decision threshold is supplied, so analytical detection cannot establish one.

11

Background reading: [FDA ICH Q2\(R2\): background definitions of detection and quantification limits, not a clinical reporting policy](#). The conversation is an original fictional teaching case.

Find the move you need

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

01 Accessioning and unambiguous identifiers

Repeat the request: You are asking about A341.

Avoid guessing: I cannot assume a digit transposition.

Separate checks: Caller identity and receiving authority both need the applicable checks.

Full phrase set: pages 7-8 | Dialogue: pages 10-11

02 Specimen suitability and recollection language

Identify the request: This message concerns test request Q6.

Correct the misunderstanding: It does not mean the patient has a low result.

Avoid blame: I cannot attribute this to a collection error.

Full phrase set: pages 16-17 | Dialogue: pages 19-20

03 Quality-control status and reportable results

Acknowledge completion: The analytical run has finished.

Correct the inference: Instrument completion does not make the report final.

Refer the review: The laboratory lead needs to resolve the review.

Full phrase set: pages 25-26 | Dialogue: pages 28-29

04 Reagents, instruments, and service coordination

Identify the instrument: I am checking the status of analyzer H2.

Separate the implication: That does not confirm return to service.

Keep repair findings separate: The visit end does not tell me what repair was completed.

Full phrase set: pages 34-35 | Dialogue: pages 37-38

Keep the conversation productive

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

05 Turnaround times and realistic updates

Identify the specimen: This timing enquiry concerns T5.

Define the service interval: The quoted two hours run from receipt to reporting.

Name the delay factor: A backlog is affecting estimates.

Full phrase set: pages 43-44 | Dialogue: pages 46-47

06 Preliminary, final, and amended reports

Open the correction: I need to correct my earlier description.

State the correct status: The ten-o'clock report was preliminary.

Avoid inventing an amendment: No later amended report is supplied here.

Full phrase set: pages 52-53 | Dialogue: pages 55-56

07 Critical notifications and acknowledged handoffs

Identify the notification: This is Lumen laboratory with a critical-result notification for verified case C7.

Attribute the designation: Lumen designates this result critical under its own criteria.

Repeat the whole value: Potassium, six point four millimoles per liter.

Full phrase set: pages 61-62 | Dialogue: pages 64-65

08 Referral laboratories and outstanding work

Identify the send-out: This handover concerns X8.

Name the missing milestone: Referral accession acceptance is not confirmed.

Accept explicitly: I accept responsibility for following up X8.

Full phrase set: pages 70-71 | Dialogue: pages 73-74

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

acceptance acknowledgment	obtain acceptance acknowledgment	69
acceptance limit	apply the approved acceptance limit	23
accession	check the accession	5
accession acceptance	confirm accession acceptance	68
accession log	consult the accession log	5
accession number	read the accession number	5
accuracy	distinguish accuracy from precision	24
acknowledgment	obtain acknowledgment	59
action owner	name the action owner	69
addendum	distinguish an addendum	51
aliquot	identify an aliquot	15
amended report	distinguish an amended report	50
analyte	name the analyte	14
analytical run	complete the analytical run	23
analyzer	identify the analyzer	32
audit trail	preserve the audit trail	6
authorized guidance	obtain authorized guidance	15
authorized recipient	identify the authorized recipient	59
authorized reviewer	consult the authorized reviewer	24
backlog	explain the backlog	41
bias	investigate documented bias	24
calibration	document calibration	33
caller identity	verify caller identity	6
case match	establish the case match	5

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

chain of custody	preserve chain of custody	68
clarification request	raise a clarification request	6
clinical action	separate clinical action	60
clinical interpretation	refer clinical interpretation	15
closed-loop communication	complete closed-loop communication	59
clotted specimen	describe a documented clotted specimen	15
coefficient of variation	report the CV with its basis	24
collection container	verify the collection container	14
collection method	confirm the collection method	14
collection time	distinguish collection time	41
communication correction	make a communication correction	50
communication discrepancy	resolve the communication discrepancy	60
complete message	repeat the complete message	60
confidentiality	maintain confidentiality	6
confirmed deadline	distinguish a confirmed deadline	41
confirmed information	relay confirmed information	42
contact extension	confirm the contact extension	60
contamination	investigate documented contamination	15
continuity	maintain continuity	69
control material	identify control material	23
control result	review a control result	23
correction trail	preserve the correction trail	51
corrective maintenance	document corrective maintenance	33
courier	contact the courier	68

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

critical notification	document the critical notification	59
critical result	communicate a critical result	59
current report	identify the current report	50
decimal point	pronounce the decimal point	59
delay notice	issue a delay notice	42
delivery confirmation	retain delivery confirmation	68
detection limit	distinguish detection from quantification	50
dilution factor	verify whether the factor was applied	33
disclosure	authorize disclosure	6
discrepant repeat	correct a discrepant repeat	60
dispatch record	consult the dispatch record	68
distribution record	check the distribution record	51
document history	preserve document history	51
downtime	report downtime	32
elapsed time	calculate elapsed time	41
end point	define the end point	41
equipment identifier	quote the equipment identifier	32
escalation route	use the escalation route	42
estimate	qualify the estimate	41
estimated completion	qualify estimated completion	42
expectation reset	make an expectation reset	42
expiry date	check the expiry date	33
explicit correction	provide an explicit correction	51
final report	confirm the final report	50

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

finding avoid inventing a finding	51
follow-up ownership transfer follow-up ownership	69
handover complete the handover	69
hemolysis report documented hemolysis	15
identifier mismatch report an identifier mismatch	5
identity resolution complete identity resolution	6
incoming technician identify the incoming technician	69
instrument status confirm instrument status	32
insufficient quantity document insufficient quantity	14
interpretation refer interpretation appropriately	51
inventory status check inventory status	33
issue time quote the issue time	50
laboratory information system consult the LIS	6
laboratory milestone confirm the laboratory milestone	69
local critical criteria apply local critical criteria	60
lot number verify the lot number	33
maintenance log consult the maintenance log	32
manifest check the manifest	68
measured concentration distinguish measured concentration	14
millimoles per liter say millimoles per liter	59
minimum necessary information limit information sharing	6
notification record complete the notification record	60
notification time record the notification time	60
numerical value correct the numerical value	59

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

operational readiness	confirm operational readiness	33
out-of-control condition	report a documented out-of-control condition	24
outgoing technician	support the outgoing technician	69
outstanding enquiry	transfer the outstanding enquiry	68
outstanding review	identify outstanding review	33
parcel delivery	confirm parcel delivery	68
patient identifier	verify patient identifiers	5
patient result	avoid inventing a patient result	14
pending accession	report pending accession	69
pending review	retain pending review	23
potassium	state the potassium result	59
preanalytical phase	discuss a preanalytical issue	15
precision	discuss analytical precision	24
preliminary report	identify the preliminary report	50
preventive maintenance	schedule preventive maintenance	33
quality control	review quality control	23
quality review	await quality review	23
quantification limit	retain the below-quantification qualifier	51
quantity not sufficient	clarify a QNS message	14
queue status	report queue status	42
quoted interval	explain the quoted interval	41
random variation	assess random variation	24
read-back	obtain a complete read-back	59
reagent	identify the reagent	32

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

reagent lot record the reagent lot	32
receipt time confirm receipt time	41
receipt-to-report interval explain the receipt-to-report interval	42
receiving authority verify receiving authority	6
receiving log check the receiving log	5
recipient acknowledgment obtain recipient acknowledgment	51
recipient role record the recipient role	60
recollection request recollection	14
recollection request relay the recollection request	15
referral laboratory contact the referral laboratory	68
referral reference request the referral reference	68
reissue verify the reissue status	51
rejection reason relay the rejection reason	15
release authorization obtain release authorization	23
report discrepancy clarify the report discrepancy	51
report issue time state the report issue time	60
report status state report status	23
report version check the report version	50
reportable result distinguish a reportable result	23
reporting time confirm reporting time	41
requisition review the requisition	5
restart authorization obtain restart authorization	32
result commitment avoid an unsupported result commitment	42
result date avoid inventing a result date	69

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

return to service confirm return to service	32
review outcome request the review outcome	24
revision reason state the revision reason	51
secure channel use a secure channel	6
send-out track the send-out	68
sender confirmation give sender confirmation	60
service provider contact the service provider	33
service report review the service report	32
service standard clarify the service standard	42
service visit record the service visit	32
shift investigate a control shift	24
source record consult the source record	6
specimen identify the specimen	14
specimen identifier confirm the specimen identifier	5
specimen requirement consult specimen requirements	15
specimen suitability assess specimen suitability	14
specimen volume distinguish specimen volume	14
stability check stability requirements	15
standard deviation express a difference in SD units	23
start point name the start point	41
status confirmation request status confirmation	33
status label correct the status label	50
status update give a status update	24
supersede supersede the preliminary version	50

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

superseded version retain the superseded version appropriately	50
test request quote the test request	14
time reference preserve the time reference	42
tracking number verify the tracking number	68
transcription error investigate a transcription error	5
transport condition confirm transport conditions	15
transport interval distinguish the transport interval	42
transport milestone distinguish a transport milestone	69
transposed digits avoid assuming transposed digits	5
trend identify a control trend	24
turnaround time clarify turnaround time	41
unavailable retain unavailable status	32
unit of measurement retain the unit of measurement	59
unverified reference retain the unverified reference	6
update commitment separate an update commitment	42
update time preserve the update time	69
verbal correction make a verbal correction	60
verification complete verification	5
verification check record a verification check	33
verified identity preserve verified identity	59
version control preserve version control	50
withheld result preserve withheld-result status	24
work queue check the work queue	41
z-score calculate the control z-score	23

Original cases. Clear boundaries.

These are original fictional teaching conversations, not transcripts of real workers. Names, figures, organizations, and incidents are invented. Sources support terminology and communication aims, not the fictional results or an endorsement of this book.

[1] US Bureau of Labor Statistics. Clinical Laboratory Technologists and Technicians.

[Open the original source](#)

Occupational context for specimen work, equipment, records, and communication. No salary forecasts or qualification advice are reproduced. Checked 10 October 2026.

[2] World Health Organization. Laboratory Quality Management System: Handbook, publication overview.

[Open the original source](#)

Overview of laboratory quality-management scope. Fictional procedures and examples are original, not reproductions of the handbook or current accreditation rules. Checked 10 October 2026.

[3] Agency for Healthcare Research and Quality. Check-Back or Repeat-Back.

[Open the original source](#)

Communication reference for recipient repetition and sender confirmation. The critical-result scenario supplies its own fictional value and local designation. Checked 10 October 2026.

Phil Stilwell

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